MEDICAL TECHNOLOGY LICENSURE EXAM REVIEW - **HTMLE**

MEDICAL TECHNOLOGY LAWS & ETHICS

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Notes by: Xiao - The Conqueror of Demons, The Vigilant Yaksha, & Alatus, the Golden-Winged King

REVIEW!!!

1. A hospital-based clinical laboratory, applied as a training facility to medical technology interns last 2003. Which among the following institution is in-charge of the approval of the said application?
   a. CHED
   b. BRL
   c. DEP-ED
   d. DOH

2. What is considered to be as an important element in the training of medical technology interns?
   a. Volume of specimen to be examined
   b. The guidelines for extension/incentives from the staff/CI
   c. Their schedule to fit with the laboratory supervisor's schedule
   d. Their off duty in a week to ensure they have time to relax and be ready for the next shift

3. If a student had been an intern in 2004, how many months had he/she spent for internship?
   a. 6 months
   b. 12 months
   c. 8 months
   d. 10 months

4. It is the PRC Modernization Act of 2000
   a. RA 8981
   b. RA 9482
   c. RA 9165
   d. RA 5527
   e. RA 7719

5. Who recommends the approval of schools of medical technology in 5527?
   a. Director of BRL
   b. Board of Medical Technology
   c. Council of Medical Technology
   d. Secretary of DOH
   e. Secretary of DepEd

6. How many water testing laboratories can a sanitary engineer manage?
   a. 1
   b. 2
   c. 3
   d. 4
   e. 5

7. Which sections of RA 5527 were amended in RA 6148?
   a. 15, 16, 21
   b. 18, 20, 21
   c. 16, 18, 20
   d. 16, 21, 22
   e. None of these

8. Which of the following sections is amended by PD 1534?
   I. Accreditation of school of medical technology and training laboratory
   II. Issuance of certificate of registration
   III. Council of medical technology
   IV. Qualifications of examiners

   a. 1 and 2
   b. 1 and 4
   c. 1, 3, and 4
   d. All of these
   e. None of these

9. Currently, who suggest qualified persons to become members of the board
   a. PRC commissioners
   b. CHED commissioners
   c. President of the Philippines
   d. President of Accredited Organizations

10. He was the first pathologist of John Hopkin's Hospital
    a. Douglas
    b. Todd
    c. Flexner
    d. Welch

11. It was the first medical technology school in the Philippines
    a. AUP
    b. CEU
    c. FEU
    d. UST

12. Who is the executive officer of the board in PD 223?
    a. Chairman of the board
    b. Commissioner of PRC
    c. Chairman of the council
    d. Commissioner of Civil Service

13. The number of medical technologist to be hired in a laboratory and workload under the provision of AO 2007-0027
    a. 1:20
    b. 1:500
    c. 1:800
    d. None of these

14. The National Blood Services Act: **RA 7719**

15. Organ Donation Act of 1991: **RA 7170**

16. Philippine Dangerous Drugs Act of 1972: **RA 6425**

17. Philippine Dangerous Drugs Act of 2002: **RA 9165**

18. PRC Law: **PD 223**

19. Father of the profession: **Nardito Moraleta**

20. Father of PAMET: **Crisanto Almario**

21. First Manila City Health Officer: **Dr. Mariano Icasiano**

22. First Medical Technology Graduate: **Dr. Jesse Umali**

23. It is a requirement before HIV testing can be performed on a patient's specimen:
    a. Chain of Custody
    b. Informed consent
    c. Medical clearance
    d. Request
    e. All of these

24. It is known as the Medical Technology Act of 1966
    a. RA 1517
    b. RA 1638
    c. RA 5527
    d. RA 4688
    e. None of these

25. Under the provision of AO 156, when is the renewal period of the blood bank?
    a. On the first day of November until the end of December
    b. First day of January to last day of February
    c. First three months of the preceding year
    d. Last day of the year

DEFINITIONS OF MEDICAL TECHNOLOGY	
Heinemann	"The application of the principles of natural, physical, & biological sources to the performance of laboratory procedures which aid in the diagnosis & treatment of disease"
Ana Fagelson	"The branch of medicine concerned with performance of laboratory determinations & analyses used in the diagnosis & treatment of disease & the maintenance of health."
Walters	"Health profession concerned with performing laboratory analyses in view of obtaining information necessary in the diagnosis & treatment of disease as well as in the maintenance of good health."
RA 5527	"An auxiliary branch of lab medicine which deals with the examination of tissues, secretion & excretion of the human body & body fluids by various electronic, chemical, microscopic & other medical laboratory procedures or techniques either manual or automated which will aid the physician in the diagnosis study & treatment of disease & in the promotion of health in general"
Clinical Laboratories	Facilities that perform chemical & microscopic examinations of various body fluids Classified as: - Government or private - Free standing or Hospital-based **TYPES OF HOSPITAL-BASED CL** Small size hospital: <100 beds; performs routine procedures Medium size hospital: 100-300 beds; does routine tests including complicated procedures Large size hospital: >300 beds; handles large volume of work & perform complex tests
Pathologist	Licensed physician specialized in Pathology (Anatomic/Clinical/Both) certified by the Philippine Board of Pathology; head/director of a clinical laboratory
Pathology	Practice of medicine which contributes to diagnosis, prognosis & treatment through knowledge gained by laboratory applications of the biologic, chemical or physical sciences to man or material obtained from a man ***Anatomic Pathology***: diagnosis or confirmation of diseases through autopsy examination and cellular differentiation of autopsy and surgical tissues ***Clinical pathology***: specializes in chemical, microbiological, and hematologic procedures
Medical Technologist	Has a baccalaureate degree from a CHED accredited school; completed a clinical training in an accredited training laboratory by the Bureau of Health Facilities & Services (BHFS)/CHED; & passed the licensure exam administered by the Board of MT

Employment Opportunities for MT	
Laboratory	Preparatory to medical school
Research	QC specialist
Academe	Drug/chemical company
Abroad	
Personal traits of a Medical Technologist	
Physical stamina	Good intellect
Good eyesight	Aptitude for biological sciences
Normal color vision	Caring attitude
Manual dexterity	
MT Special Characteristics	
Good communication skills	Accurate
Ability to relate to others	Skillful
Service oriented-dedication	Emotional maturity
Patience	X-factor
Honest	

THE ORIGIN

DATE	EVENT/CONTRIBUTION	PERSON/S
1500 BC	**Vivian Herrick**: traces MT to the discovery of *Taenia* & *Ascaris* as mentioned in some writings	**Ebers Papyrus**: An early writing Containing treatment of disease & the description of the three stages of hookworm infection
600 BC	**Urine analysis**: Oldest laboratory procedure	Hindu physician describing urine (polyuria & sweet urine)
460 BC	**Hippocrates**: Father of medicine	Described 4 "humors" or body fluids in man Black bile, blood, phlegm, yellow bile: felt to be the source of person's disposition & disease in ancient times Urine is also considered as a composite of the humors
	Chinese: Practice immunization by inhaling powder made from smallpox scabs to develop immunity to the disease	
	Romans: Developed laboratory tools such as forceps, scalpels, specula, and surgical needles	
	Indians: Practice of toxicology (agadatantra) was mentioned in the Ayurveda classics	
	Rufus of Ephesus: First describe hematuria by correlating the presence of blood to the physiological functions of the kidney	
1096 – 1438	Believed that MT started in medieval period	"An Introduction to the Profession of Medical Technology" book **Ruth Williams**: MT started in medieval period when descriptions to urine begun & U/A was a fad
MEDIEVAL ERA	Used the microscope to investigate the causes of diseases	**Athanasius Kircher**
	Published the *micrographia* which featured illustrations of his observations using a microscope	**Robert Hooke**
	Developed the gravimetric analysis of urine	**Jean Baptiste van Helmont**
	Described the protein in urine	**Frederick Dekkers**
	Performed the first blood transfusion in animals	**Richard Lower**
	Described the process of coagulation	**William Hewson**
	Developed the yeast test for sugar in diabetic urine	**Francis Home**
	Identified sugar in the blood and urine of diabetics	**Matthew Dobson**
	Anna Fagelson: dated back the start of MT to the death of a laboratory worker, Alessandra Giliani (through nosocomial infection)	**Dr. Mondino de Luzzi**: "father of anatomy" **Alessandra Giliani**: assistant to Mondino de Luzzi 1628
1628 – 1694	Greatest early microscopist "founder of pathology" because of his contributions to embryology & anatomy	**Marcello Malphigi**
1632 – 1723	Invented & improved compound microscope	**Anton Van Leeuwenhoek**
1800	Investigated certain diseases through laboratory methods	**Dr. William Occam**
1847	One of the youngest medical specialist Coined the term "leukemia"	**Rudolph Virchow**
1848	Performed the 1st quantitative test for sugar	**Herman Fehling**
	Revolutionized medicine through the development of anatomical pathology (autopsy & biopsy)	**Dr. Baron Karl von Humbeldt**
19th century	Discovery of aniline dyes	
	Country that revolutionized MT profession	MT became known in Germany, spread & developed in Greece, Japan, Turkey, England and **United States**

FIRST LABORATORIES

Event/contribution	
First laboratories	Laboratory in the **University of Michigan** - well equipped & the largest at that time (not a clinical laboratory but a chemical lab) 1854: **Medical College of St. Bartholomew** 1858: **University College at Cambridge**
Started the first laboratory instruction	**Dr. Douglas**
First clinical laboratories	**John Hopkin's Hospital** - was fully equipped; with the cost of fifty dollars & occupied a 12 x 12 size room (1896) University of Pennsylvania (William Pepper Laboratory): 1896

MEDICAL TECHNOLOGY IN THE UNITED STATES

occurred in late 19th century

DATE	EVENT/CONTRIBUTION	PERSON
1878	Established a laboratory at Bellevue Hospital Medical College	**Dr. William Welch**
1885	First professor of pathology in John Hopkins University	
	Initiated the use of laboratory animals for experimentation in his diagnosis	**Burdon Sanderson** obtained significant results in his works in medicine by applying laboratory medicine methods
1896	Opened the first clinical laboratory at JHH	**Dr. William Osler**
1896	**1900 census** of US showed that 100 males were employed as laboratory technicians in US employment increased in 1920 to 3,500 & in 1922 3035 hospitals had Clinical Laboratories	
	1st Pathologist in JHH	**Dr. Simon Flexner**
1908	Wrote "***A Manual of Clinical Diagnosis by Laboratory Methods*** (1st – 6th Ed.)"	**Dr. James C. Todd**
1915	A law in Pennsylvania was passed requiring all hospitals to have a lab and employ a full time lab tech	
1921	Organization of Denver Society of Clinical Pathologists was established	
1923	The first school for training laboratory workers was established at **University of Minnesota** & 1st to confer a degree program	
1936	Organization of **American Board of Pathology** was established	
1940	US required a two-year collegiate education and 12- month actual laboratory training for the preparation of the practice	
1950	Standard curriculum for the medical technology was formalized	
World War I	Growth of MT as demands for technicians increased	
World War II	Marked effect on lab medicine in the following manner: - closed system collection of blood - advancement of instruments	

THE GROWTH OF CL HAPPENED IN THIS ORDER:

1. Pathology
2. Clinical Microscopy
3. Studies of Red Blood Cells
4. Physiological Chemistry
5. Bacteriology

MEDICAL TECHNOLOGY IN THE PHILIPPINES

DATE	QUESTION	EVENT/PERSON
October 20, 1944		US bases were built in Leyte
January 9, 1945	First medical facility	26th Medical Laboratory
	Introduced the first clinical laboratory	6th US Army
	Location	Quiricada St., Sta. Cruz Manila (Now: Public Health Laboratory)
Feb 1945		Health care training among civilian Filipinos started
June 1945	Endorsed to whom?	National Department of Health
		Dr. Alfredo Pio de Roda assisted by **Dr. Mariano Icasiano** reorganized the lab
	Name of the laboratory	Manila Public Health Laboratory
October 1, 1945		A medical laboratory was born under a Filipino leader
1947	Trainers of the medical technicians	**Dr. Alfredo Pio de Roda** and **Dr. Prudencia Sta. Ana**
	Composition of the trainees	High school and paramedical graduates
1954	Duration of the training	6 months with certificate

FORMAL PHILIPPINE MEDICAL TECHNOLOGY EDUCATION		
DATE	**EVENT/CONTRIBUTION**	
1954	First school that offered 4-year MT curriculum	**Philippine Union College** (Adventist University of the Philippines) of Baesa, Caloocan, Rizal
	First school of medical technology in the Philippines	**Manila Sanitarium**
1956	First PUC MT graduate	**Dr. Jessie Umali**
1957-1958	School that offered MT as an elective course among 4th and 5th year pharmacy students	**University of Santo Tomas**
June 17, 1957	Permit was issued to UST to offer 1st - 3rd year	
1960	Centro Escolar University	
June 1960	Permit was issued to UST to offer internship program	
June 14, 1961	Full recognition of UST MT course	
1962	Far Eastern University	

MT ORGANIZATIONS

PASMETH (1970)

- Philippine Association of Schools of Medical Technology & Public Health
- Aims to maintain the highest standard of MT education & foster closer relations among these schools

QUESTION	
Date organized	**May 13, 1970**
Organized by	**Director Narciso Albarracin** appointed **Dr. Serafin Juliano** & **Dr. Gustavo U. Reyes** to organize an association of Deans/Heads of Schools of MT & Hygiene
Date of registration with the Securities & Exchange Commission (SEC)	Oct. 6, 1985 Thru the Committee on Legislation chaired by **Mr. Cirilo S. Cajucom** with the help of a legal counsel, **Atty. Dexter Bihis**
Place & date of the 1st organizational meeting was held	June 22, 1970 at University of Santo Tomas
First set of officers	President = **Dr. Gustavo Reyes** Vice-President = Dr. Serafin Juliano Sec./Treas. = Dr. Velia Trinidad PRO = Dr. Faustino Sunico
Place & date of the 1st annual meeting	May 17, 1971 at UST
Date when the 1st set of officers reelected for a 2nd term	April 30, 1972 President = **Dr. Gustavo Reyes** Vice-President = Dr. Claro Cabrera Sec./Treas. = Dr. Elvira Silva PRO = Dr. Faustino Sunico
Other PASMETH presidents	

1973-1974	Dr. Ibarra Panopio
1974-1977	Dr. Angelita G. Adeva
1977-1980	Dr. Elizabeth M. del Rio
1980-1981	Dr. Gustavo Reyes
1981-1982	Dr. Claro D. Cabrera
1982-1983	Dr. Elizabeth M. del Rio
1983-1984	Dr. Norma V. Lerma
1984-1985	Dr. Vicencio T. Torres
1985-1988	Prof. Nardito Moraleta
1988-1995	Dean Norma N. Chang
1996-2000	Prof. Rodolfo R. Rabor
2000-2002	Dean Nini F. Lim
2002-2010	Dean Zenaida C. Cajucom
2010-2012	Magdalena Natividad
2012-present	Dean Bernard Ebuen

THE PASMETH SEAL

- Circular with yellow lines in a field of gold emblazoned with the words PHILIPPINE ASSOCIATION OF SCHOOLS OF MEDICAL TECHNOLOGY / PUBLIC HEALTH in black encircling it.
- In the middle of the circle is a diamond-shaped figure in yellow encasing a microscope in black and the letters PASMETH across the microscope in green.
- Beneath the microscope is the year 1970.

QUESTION	
Represents the continuity of learning and the never-ending quest for excellence in the field of academe	Circle
Represents the four objectives of the Association 1. To encourage a thorough study of the needs and problems of Medical Technology and Public Health Education and to offer solutions to them. 2. To work for the continuous development of Medical Technology and Public Health Education in order that the profession will be of maximum service to the country. 3. To take a united stand in matters which affects the interests of Medical Technology and Public Health Education, and 4. To seek advice, aid and assistance from any government or private entity for the fulfillment of the aims and purposes of the Association	Four corners of the diamond
Represents the field of Medical Technology and Public Health?	Microscope
1970 symbolizes	Year the Association was founded

THE PASMETH HYMN

This We Say
As we hone our minds to see the truth
We open up our hearts for our fellowmen
That they be given the duty that we are
And as we unite
We realize that in each other's arms
Through the darkest nights
We can see the light of the deathless stars
To our Motherland we offer thee
The fruit of our toil
As we climb the steep

We will strive to keep
Our blood in our soil
And as we fulfill our destiny
We'll see our vision through
As we pledge to thee
That in PASMETH
We shall oath our hearts in truth
And may God enact our sacred pact
To the world, this we say
Let progress be in the breast of learning
And may they never part

PAMET (PHILIPPINE ASSOCIATION OF MEDICAL TECHNOLOGISTS)

- Aims to standardize & to give dignity to the medical technology profession

EVENT CONTRIBUTION	
Date PAMET was organized	September 15, 1963
Person who organized	**Mr. Crisanto G. Almario** (Pharmacist by profession)
Place where it was organized	San Lazaro Hospital in Sta. Cruz, Manila
The Father of PAMET	**Mr. Crisanto Almario**
Date and place of the 1st convention & election of officers	September 20, 1964 at Far Eastern University
The 1st PAMET President	**Mr. Charlemagne Tamondong**
Date of registration with the Securities & Exchange Commission (SEC)	October 14, 1969 with Reg. No. 39570, during the presidency of Mr. **Nardito Moraleta**

THE PAMET INSIGNIA

QUESTION	
Symbolizes the continuous involvement and integration of practice and education	**Circle**
Symbolizes the trilogy of Love, Respect, and Integrity	**Triangle**
The color of health	**Green**
Symbolizes the science of MT profession	**Microscope & Snake**

LEGACY OF PAMET PRESIDENTS		
YEAR	**PRESIDENT**	**LEGACY**
1963-1967	Charlemagne T. Tamondong	Emergence of the Profession
1967-1970	Nardito D. Moraleta (Father of the profession)	Professional Recognition *Cebu was the first provincial chapter
1970-1971, 1973-1977	Felix E. Asprer	Legislative Agenda *PD 6138 & PAMET accreditation by PRC
1971-1973	Bernardo T. Tabaosares	Celebration of the Practice *Medtech Week & Davao chapter
1973	Angelina R. Jose	Career Advocacy *Professional tax
1977-1981	Venerable C.V. Oca	Educational Enhancement *PAMET quizbee
1982-1991	Carmencita P. Acedera	Image Building
1992-1996	Marilyn R. Atienza	Proactivism
1997 – 2000	Norma N. Chang	International Leadership
2001-2002, 2005-2006	Agnes B. Medenilla	Organizational Dynamism
2003-2004	Shirley F. Cruzada	Interdisciplinary Networking
2007-2012	Leila M. Florento	Beyond Expectations
2013-2015	Romeo Joseph J. Ignacio	VOICE
2016-2020	Ronaldo Puno	
2020-Present	Rommel Saceda	

PAMET HYMN

Music: **Francis Jerota Pefanco**
Lyrics: **Hector Gentapanan Gayares, Jr.**

- From various lands races and places with grateful hearts we blend our voices
- This day to our beloved PAMET from whence unity and love cometh
- We join together in brotherhood to live up to thine ideals we should
- In fields of advancement and learning
- Thy noble goals maybe our bearing.
- Loyal and true we'll be to thee
- Beloved PAMET this we say, for service to God and humanity
- With joy we sing for thee till eternity

DEFINITION OF TERMS	
Law	Set of rules or orders imposed on all members of the community
Republic Act	A piece of legislation enacted as a policy that may govern the conduct and behavior of an individual or group of persons and uphold the principles of the Constitution.
Presidential Decree	Connotes the judgment of the court; it includes the proclamations of the legal consequence of the facts or actions in connection with the professional practice carried out or decided by the court in its finality.
Executive order	It contains the president's directive implementing or interpreting certain constitutional provisions or treaties.
Senate bill	A piece of proposed legislation that has been issued or was modified by the Congress or the Senate; can become a law after its passage by the Senate and House of Representatives and the approval of the President of the Philippines
Ethics	Also known as moral philosophy, is a branch of philosophy that addresses questions about morality – that is, concepts such as good and evil, right and wrong, virtue and vice, justice, etc.
Bioethics	The study of controversial ethics brought about by advances in biology and medicine. Bioethicists are concerned with the ethical questions that arise in the relationships among life sciences, biotechnology, medicine, politics, law and philosophy.

PERTINENT LAWS THAT APPLY TO MEDICAL TECHNOLOGISTS	
Today, the existing law of the land mandates the professional practice of medical technology. The rules and regulations pertaining to the same should be complied with. Other pertinent laws are the existing Republic Acts that were formulated for the benefit of medical technology professionals.	
The Professional practice of medical technology in the Philippines is governed by the following:	
LABOR LAW "*Omnibus Implementing Rules and Regulations*"	- Considers the existence of employee-employer relationship - A medical technology professional is entitled to the benefits extended to him/her by an employer-employee relationship - In case of work grievance and in times of abuse, a medical technology professional may consult and invoke the provision of the Criminal Law - Laws or orders governing labor relations, social legislation, conditions of employment, human resource development programs, health, safety, benefits, & pre- & post-employment.
CRIMINAL LAW *embodied in the Revised Penal Code*	- A system of laws or orders governing punishable acts or omissions & their corresponding penalties - A medical technology professional who commits an offense can never be denied due process and equal protection
CIVIL LAW OF THE PHILIPPINES	- The laws or orders governing persons, property, ownership, obligations, succession, interstate proceedings, & contracts, family, & marriage - A medical technology professional is protected because of the contracts and obligations attached to his/her medical technology professional practice
CONSTITUTION OF THE PHILIPPINES	- Since a medical technology professional is a citizen (whether natural born or naturalized) of the Philippines, he/she enjoys the Bill of Rights.

LEGAL ISSUES AND CONCERNS IN THE PRACTICE OF MEDICAL TECHNOLOGY IN THE PHILIPPINES

1. The right to perform the profession with minimal supervision by a registered Pathologist.
2. The right to enjoy the privilege and prestige of being a medical technologist.
3. The right to have just compensation.
   - Just compensation means fair compensation and salary including the benefits received by an employed medical technologist which is commensurate to the service he/she renders as a professional. This requirement was included in the Labor Law.
   - Regular working hours: *should not be more than 8 hours of any day & not more than 40 hours in any one week* (*Regular Working Hours, Sections 5, Rule 1A, Book III*)
4. The right to have hazard pay & other benefits.
   - -Every working medical technologist is entitled to receive hazard pay and other benefits extended to any person being employed in a clinical, hospital-based, or school-based laboratory. Hazard pay is usually given monthly or quarterly, depending on the working conditions.
5. The right to have continuous professional growth and development.
   - As a member of various professional organizations, he/she is required to attend seminars or workshop for professional advancements and updating
   - A medical technologist who attends a seminar can receive a salary and can declare his/her attending an official engagement as working time, if the conditions are met pursuant to the provision of Omnibus Implementing Rules and Regulations of the Labor Code of the Philippines
   - Attendance in lectures, meetings, or training programs and other similar activities shall not be counted as working time if all the following conditions are met:
     - Attendance is outside the employee's regular working hours
     - Attendance is, in fact, voluntary, and
     - The employee does not perform any productive work during such attendance (*lectures, meetings, and training programs in Section Book III, Rule 1*)
6. The right to have conducive work place and good working conditions.
   - Good working conditions is more pronounced in the provision of the labor Code of the Philippines. One of these provisions is as follows:
     - Every employer shall give his employees, regardless of sex, *not less than one hour for regular meals*, except in cases of a meal period not less than twenty minutes may be given by the employer provided that shorter meal period is credited as compensable hours of the employee

- Along with the granting of these rights, however, are equivalent and corresponding duties, responsibilities, and obligations.
- The medical technologist should observe all of the rules and regulations, submit himself/herself to authorities or superior officials, observe professionalism; do the assigned task with accuracy, diligence, and reliability; and obey the lawful order of superior officials.
- As stated in the Civil Code of the Philippines:
  - Every person is obliged to give something is also obliged to take care of it with the proper diligence of a good father of a family, unless the law or the stipulation of the parties requires another standard of care (*Nature and Effects of Obligation in Chapter 2, Book IV - Obligation and Contracts, NCC*)
  - Every person must, in the exercise of his rights and in the performance of his duties, act with justice, give everyone his due, & observe honesty, & good faith (*Article 19, Chapter 2, Civil Code of the Philippines*)

THE PHILIPPINE MEDICAL TECHNOLOGY LAW

- RA 5527
- RA 6138
- PD 498
- PD 1534
- Senate Bill 2722
- Senate Bill 2473

RA 5527
"THE PHILIPPINE MEDICAL TECHNOLOGY ACT OF 1969"

- Approved: **June 21, 1969**

Sections	
	Preliminary Provisions
1, 2	Title and definition
	Functional Provisions
3,4,5	Council of Medical Technology
6	Minimum Required Course
7-12	Board of Medical Technology
13	Accreditation of Schools of Medical Technology
14	Inhibition to the Practice of Medical Technology
15-20	Board of Examination
21-26	Certification
27	Foreign Reciprocity
	Concluding Provisions
28	Roster of MT
29	Penal provision
30	Separability Clause
31	Repealing Cause
31	Effectivity

SECTION & TITLE		
Section 1 - **Title** Medical Technology Act of 1969		
Section 2 - **Definition of Terms**		
Section 3 - **Council of Medical Technology Education**	Chairman of the Council	- Secretary of Education
	Vice-Chairman	- Director of the Bureau of Research and Laboratories (BRL)
	Members	- Has 7 members - Chairman and the 2 members of the Board of MT, (3) - Dean of the of the Institute of Hygiene of the University of the Philippines (1) - A representative of the deans or head of the private schools (1) - President of PAMET and the Philippine Society of Pathologists (2)
Section 4 - **Compensation & traveling expenses of Council members**		
*The Council of Medical Technology Education has already been replaced by the Technical Panel in Medical Technology Education under CHED		
Section 5 - **Functions of the Council**	Functions of the Council	- Recommend required curriculum - Recommend the number of students to be enrolled in MT in each school - Approve MT schools - Require MT schools to make annual report - Inspect MT schools - Certify admission to an undergraduate internship training - Recommended approval of refresher course to 3rd time failures
Section 6 - **Minimum required course**	Minimum required course	- 3 years academic and 1-year internship - General, core and professional subjects
	Authorized to change, remove & add subjects	- Council
	Approves the changes, remove, & add subjects	- Secretary of Education Case: one of the Board of Medical technology who is a pathologist met a car accident and died on the same month when his appointment was approved 2 years ago. Another pathologist will be appointed to replace him. - Who will appoint the successor? ***President of the Philippines*** - How long will the newly appointed member serve his term as mandated in RA 5527? ***Only 1 year left since the past pathologist had already spent his 2-year term.*** - How long will the newly appointed member serve his term as mandated in PD 498? ***3 years***

Section 7 - **Medical Technology Board of Examiners**	Chairman of the Board	- Pathologist
	Recommends the chairman of the board	- Philippine Society of Pathologists (PSP)
	Duration of the term of office of the chairman	- 3 years
	Members of the Board	- 2 RMTs
	Duration of the term of office of the members	- 2 years, & 1 year, respectively
	Recommends the members of the board	- PAMET
Section 8 - **Qualifications of Examiners**	Qualifications of the Board	- Filipino Citizen - Good Moral character - Qualified pathologist/ Medical Technologist - At least 10-year experience prior to appointment (reduced to 5 years) - Not a faculty member of any MT school prior to appointment
Section 9 - **Executive Officer of the Board**	Serves as the executive officer of the Board	- Civil Service Commissioner
Section 10 - **Compensation of Board**		
Section 11 - **Functions of the Board**	- Administer the provisions of this Act - Administer oaths - Issue, suspend, revoke COR of MT - Look into conditions the practice of MT, adopt measure deemed proper for the maintenance of good ethical & moral standards - Investigate violations of this act - Draft rules and regulations necessary to carry out the provision of this act	
Section 12 - **Removal of Board Members**	Authorized to remove members of the Board	- President of the Philippines
	Grounds for the removal of the board	- Neglect of duty - Incompetency - Malpractice or unprofessional, unethical, immoral or dishonorable conduct
Section 13 - **Accreditation of Schools of Medical technology and of Training Laboratories**	Agency that approves the accreditation of MT Schools	- Department of Education (DepEd)
	Recommends the approval of accreditation of Medical Technology Schools	- Council of Medical Technology Education
	Agency that approves the accreditation of Training Laboratories	- Department of Health (DOH)
	Recommends the approval of accreditation of Training Laboratories	- Council of Medical Technology Education
Section 14 - **Inhibition Against the Practice of Medical Technology**	Person/s exempted by the law to practice MT profession without a valid COR	- Duly **registered physician** - **Medical Technologist from other countries** called in for consultation or as visiting or exchange professors to colleges or universities *provided they are practicing the said function.* - **Medical technologists** in the service of the **US armed forces** stationed in the Philippines, *provided their services will be for the said forces only*
Section 15 - **Examination**	Schedule of examination	- Date is published in newspapers at least 30 days prior to examination (*Manila, Cebu & Davao during August or September*)
Section 16 - **Qualification for Examination**	Qualification of a person who may take the board exam	- Good health and moral character - Completed the 4-year course of MT/BS in hygiene - Paramedical graduate practicing MT for 5 years prior to the date of exam, if such performance began prior to June 21, 1969
Section 17 - **Scope of Examination**	Subjects in the Board Exam	- Clinical Chemistry (20%) - Microbiology & Parasitology (20%) - Hematology (20%) - BB & Serology (20%) - Clinical Microscopy (10%) - Histotechnique (10%)
	Authorized to change, add or	- Board of Medical Technology

	remove from list of subjects or weights	
	Authorized to approve changes in the board exam	- Council of Medical Technology Education
	Publication of the changes in the board exam	- Must be done at least 90 days or 3 months prior to the date of examination
Section 18 - **Report of Rating**	Duration for the publication of result	- Within 120 days after the board exam, the Board reports the result to the *Commissioner of the Civil Service*
	Approves the publication of result	- President of the Philippines
	A directive that institutionalized the full computerization of the MT Licensure Exam	- Executive order 200
Section 19 - **Rating in the Exam**	Conditions for examinees to pass the MT board	- At least the general average is 75% - No rating below 50% in any major subjects - Does not fail in at least 60% of the subjects computed according to their relative weights - No further examination is given to 3rd takers unless a refresher course is taken
Section 20 - **Oath Taking**	A requirement before a successful examinee can have a COR	- Oath taking

Case: An examinee for MT board was born March 30, 1991. He graduated in UP, college of Public Health. He took the board exam last September 2010 and March of 2011 but was unable to get the minimum rating to pass. He took again the Medical Technology board exam last September 3 & 4, 2011. His ratings are as follow:

Clinical Chemistry - 89%
Hematology - 84%
Blood Banking/ Serology - 49%
Clinical Microscopy - 74%
Microbiology/ Parasitology - 86%
Histopathology/ MTLE - 75%

What is the General Weighted Average? **76.5%**
Based on RA 5527, can he practice as a medical technologist? **No**
If he failed during the examination, can he take the medical technology board exam again? How? **Yes (Apply again)**

Section 21 - **Issuance of Certificate of Registration**	Conditions for the issuance of COR	- *Successful examinee* - Age: **21 years old**
	Persons who will sign the COR	- Signed by members of the board (attested by the secretary)
	Instances wherein COR without exam can be issued to BSMT/BSPH graduates locally or abroad	- Foreign curriculum has standard similar to ours - Practicing MT for at least ***3 years*** prior to the filing of application if such performance occurs prior to ***June 21, 1969***
	Instances wherein COR without exam can be issued to persons graduated from other paramedical profession	- ***Civil service eligible*** & who are actually performing MT practice for the last ***5 years*** prior to the enactment of this Act
Section 22 - **Fees**		
Section 23 - **Refusal to issue COR**	Grounds for the refusal of COR issuance	- Any person convicted by a court off any **CRIMINAL OFFENCE** involving moral turpitude, or any person "guilty" of: ○ immoral or dishonorable conduct, ○ unsound mind, or ○ Incurable communicable disease (ex. HIV/AIDS).
	Conducts the investigation	- 2 Board members with - 1 legal officer

Section 24 - **Administrative Investigation**	Grounds for the conduct of investigation	- Violations in section 29, or for unprofessional conduct , malpractice, incompetence or serious ignorance or negligence in the practice of MT
	Unanimous vote	- Three (3) of the board members have the same decisions
	Penalty of a unanimous vote	- No penalty or revocation of license (perpetual disqualification)
	Majority vote	- 2/3 of the board members have the same decision
	Penalty of a majority vote	- Penalty or reprimand or suspension *(not >2 years)*
Section 25 - **Appeal**	Addressee of the appeal	- Civil Service Commissioner
	Duration before a decision shall become final	- 30 days after his promulgation
	Addressee of the final appeal	- President of the Philippines
Section 26 - **Reinstatement, Reissue or replacement of COR**	Conditions wherein COR can be Reinstated, Reissued or replaced	- If found innocent after investigation - Upon the expiration of the period of suspension
Section 27 - **Foreign Reciprocity**		
Section 28 - **Roster of Medical Technologist**	Keeps the rooster of successful examinees	- Secretary
Section 29 - **Penal Provisions**	Violations in RA 5527	- Practicing MT w/o COR - Practice not supervised by a qualified Pathologist - Making fraudulent lab report - Refusal to display COR after given a warning - Impersonate any registrant of like or the same name - Presenting/attempting to use revoked or suspended COR - Giving to the board false evidences to get COR - Advertising being a MT without COR - Employing a non-MT/Tech in lab
Section 30 - **Separability clause**		
Section 31 - **Repealing Clause**		
Section 32 - **Effectivity**		- June 21, 1969

Case: An examinee was born to a Filipino couple on September 18, 1950. He finished his Bachelor of Science in Pharmacy in a prestigious school as Cum laude and had a short stint working in a clinical laboratory. He took the Medical Technology Licensure Examination last September 1 & 2, 1970 in Metro Manila. Assuming that he got these scores, answer the following questions:

Hematology - 91%
Clinical Microscopy - 48%
Blood Banking/ Serology - 94%
Microbiology/Parasitology - 53%
Clinical Chemistry - 93%
Histopathology/MTLE - 88%

What is his general weighted average? **79.8%**
Based on RA 5527, can he practice as a medical technologist? **No (Underage)**
If he failed during the examination, can he take the medical technology board exam again? How? **Yes (Apply again)**
In the event that the examinee failed to pass the MTLE until his three tries, can he take the board exam again? Why? **No**

RA 6138

- Approved: **August 31, 1970**, effectivity retroactive as of June 21, 1969
- Amended sections:
  - 16 = Qualification for examination
  - 21 = Issuance of COR
  - 22 = Fees

SECTION	TITLE	DETAILS
16	Qualification for examination	<table><tr><th>5527</th><th>6138</th></tr><tr><td>BSMT or Hygiene graduate & ParaMed graduate (with 5 years of experience)</td><td>BSMT or Hygiene graduate or ParaMed graduate (with 5 years of experience)</td></tr><tr><td>prior to the enactment of this Act</td><td>prior to the date of examination if such performance began prior to the enactment of this Act</td></tr></table>
21	Issuance of COR without exam	<table><tr><th>5527</th><th>6138</th></tr><tr><td>BSMT or Hygiene graduate (3-year experience)</td><td>BSMT or Hygiene graduate (3-year experience)</td></tr><tr><td>Paramed - civil service eligible (5-year experience)</td><td>Paramed - civil service eligible (5-year experience)</td></tr><tr><td>At the time of passage of this act</td><td>Prior to the filing of the application</td></tr></table>
22	Fees	10 pesos (RA 5527 = P50)
Approval		August 31, 1970
Effectivity		Retroactive as of June 21, 1969

PRESIDENTIAL DECREE NO. 498 (PD 498)

SECTION	QUESTIONS	DETAILS
	Date of approval	• **June 28, 1974**
	How many sections were amended?	• 11 2, 3, 4, 7, 8, 11, 13, 16, 17, 21, 29
2 - **Definition of Terms** *(Specifically on the practice of Medical Technology and Medical Laboratory Technician)*		
3 - **Council of Medical Technology Education, Its Composition**	Serves as the chairman of the council	• **Commissioner of PRC**
	The vice-chairman of the council	• **Chairman of the MT Board** (Pathologist)
4 - **Compensation & Travelling Expenses of Council Members**		
7 - **Medical Technology Board**	The term of office of the Board	• **3 years all**
	Recommended by	• **Professional Regulation Commission (PRC)**
8 - **Qualification of Examiners**	All four previous qualifications: • Filipino Citizen • Good moral character • Pathologist/medical technologist • At least 10 yrs. Experience prior to appointment, (reduced to 5 yrs. – deleted)	
	Number of years of affiliation by the board members to MT/PH schools	• Not affiliated for **2 years** in school
11 - **Functions and Duties of the Board**	• (c) issue, suspend & revoke COR for the practice of *med tech & lab technician*; Added: • (g) to determine the adequacy of technical staff of all clinical labs & blood banks • (h) to prescribe the qualification & training of MT as the special fields of the profession & supervise their special exam conducted by the professional organization of MT accredited by the PRC • (i) to classify & prescribe the qualifications & training of the technical staff of clinical labs as to: Chief MT; Senior MT; MT & Medical Lab Tech	
13 - **Accreditation of Schools of Medical Technology and of Training Laboratories**	Agency that approves the accreditation of Medical Technology Schools	• **DECS**
	Recommends the approval of accreditation of Medical Technology Schools	• **MT BOARD**
	Agency that approves the accreditation of training Laboratories	• **PRC** Professional Regulation Commission
	Recommends the approval of accreditation of Training Laboratories	• **MT BOARD** ○ 5527: DepEd/DOH & recommended by the Council of MT Education
16 - **Qualifications for Examination**	• Paramed practicing MT profession 5 yrs. Prior to the date of examination prior to June 21, 1965 • Deleted: minimum requirement & 1-year undergraduate internship or practical training <table><tr><th>RA 5527</th><th>RA 6138</th><th>PD 498</th></tr><tr><td>BSMT or Hygiene graduate & ParaMed graduate (with 5 years of experience)</td><td>BSMT or Hygiene graduate or ParaMed graduate (with 5 years of experience)</td><td>BSMT or BSPH graduate or paramed – 5 years of experience</td></tr><tr><td>prior to the enactment of this Act</td><td>prior to the date of examination if such performance began prior to the enactment of this Act</td><td>prior to the date of examinations, if such performance began prior to June 21, 1969</td></tr></table>	
17 - **Scope of Examination**	Clinical Chemistry……20% Microbiology & Parasitology……20% Hematology……20% Blood Banking & Serology……20% Clinical Microscopy……10% Histopath Techniques……10% **Histopathologic Techniques, Cytotechnology, Medical Technology Laws, Related Laws and its implementing rules, and the Code of Ethics*	
	Authorize to change, add or remove from the list of subjects or weights	• **BOARD OF MT**
	Approves changes in the board exam	• **PRC** ○ 5527: Council
21 - **Issuance of COR**	Persons who signs the COR	• **PRC Commissioner & 3 Board Members** ○ 5527: Board attested by its Secretary
	Instances wherein COR can be issued to MT without exam to BSMT/BSPH graduates	• COR w/out examination to: ○ BSMT/BSPH graduate practicing MT for at least 3 years prior to the filing of application ○ Paramed w/ civil service exam & **8 yrs. experience** prior to the filing of the application ▪ 5527 = 5 yrs. Prior to the enactment of this Act
	COR as medical lab technician was issued by the MT board w/out examination of he/she	• Passed the civil service exam for medical technician given on **March 21, 1964**; • Finished a **2-year** college course & has at least **1-year** of experience as medical lab technician; 1 yr. deficiency in college attainment = **(2) years of experience** maybe submitted; • Applicant has at least **10 years** of experience as medical lab technician; and • Failed to pass MT board examination but obtained a general rating of at least **70%**
29 - **Penal Provisions**	• Any person/corporate body employing/recommending a non-MT to practice MT ○ 5527: Any persons or corporate body who shall violate rules & regulation of Board or orders promulgated by it	

Senator Bill No. 2722 Engardo Angara 15th congress
Senator Bill No. 2372 Teofisto Guingona III 16th congress

PRESIDENTIAL DECREE NO. 1534 (PD 1534)

SECTION	QUESTION	DETAILS
	Date of approval	• **June 11, 1978**
	Number of sections amended	• Amended **4** sections
3 - **Council of Medical Technology Education, Its Composition**	Serves as the chairman of the council	• **Director of Higher Education**
	The vice-chairman of the council	• **Commissioner of PRC**
8 - **Qualification of the Examiners**	• He/she: is a qualified Pathologist (formerly deleted in PD 498)	
11 - **Functions and Duties of the Board**	• Subparagraph (g) and (i) of Section 11 of Republic Act No. 5527 as amended, are hereby repealed	
13 - **Accreditation of Schools of Medical Technology and of Training Laboratories**	Agency that approves the accreditation of Medical Technology Schools	• **Department of Education & Culture**
	Recommends the approval of accreditation of Medical Technology Schools	• **MT BOARD**
	Agency that approves the accreditation of Training Laboratories for MT students	• **BRL-DOH** ○ 5527: DOH ○ PD 498: PRC
	Recommends the approval of accreditation of Training Laboratories	• **MT BOARD** ○ 5527: Council of MT Education

RA 4688
CLINICAL LABORATORY LAW

SECTION	QUESTIONS	DETAILS
	It is also known as the Clinical Lab Law	
	Date of Approval	• **June 18, 1966**
	Purpose of 4688	• Prevent the operation and maintenance of substandard, improperly managed & poorly equipped clinical laboratories • The implementing guidelines of RA 4688 are contained in AO 2007-0027
1	Requirements before a clinical lab can be operational	• **LTO and Authorization** to the head of the lab
	Laboratories exempted from securing LTO	• **Gov't hospital labs** doing **routine** or **minimum lab exams** (services are extensions of gov't regional or central labs)
2	Requirements of the head of a clinical lab	• Licensed physician duly qualified in laboratory medicine & authorized by the Secretary of Health
3	Authorized to enforce the provisions of 4688	• **Secretary of Health**
4	Penalties	• Imprisonment for ***not < 1 month but not > 1 year, or by a fine of not < 1,000 but not > 5,000 pesos***, or both such fine & imprisonment, at the discretion of the court
5		• Separability clause
6		• Budget
7		• Repealing clause
8		• Effectivity

Amendments of RA 4688:

- AO 201
- 290
- 49-B
- AO2
- AO 59
- AO 2007-0001
- AO 2007-0027
- AO 2008-0007

ADMINISTRATIVE ORDER NO. 201 Series of 1973

SECTION	QUESTIONS	DETAILS
1 **Purpose and Title**	***REVISED RULES & REGULATIONS GOVERNING THE REGISTRATION OPERATION & MAINTENANCE OF CLINICAL LABORATORIES IN THE PHILIPPINES***	
2 **Scope**	• Any person, firm or corporation, operating & maintaining clinical labs that performs test from ***human beings***	
	Laboratories that are exempted from securing LTO	• Gov't hospital labs doing routine or minimum lab exams (services are extensions of gov't regional or central labs) • Labs used solely for **academic training** and/or for **purely research activities**
3 **Basic License Requirements**	Contents of the application form: (1) name, citizenship & domicile of the head of the lab; (2) place, municipality & province it is to be established (3) the name of the establishment; (4) the name, citizenship & domicile of the owner, (5) scope & nature of work to be undertaken; (6) statement of compliance of all business requirements of the applicant; & (7) tax clearance for the preceding year	
	Number of days for inspection, after the receipt for application	• Within **60 days**
	Person who conducts the inspection	• **BRL Director**/His Representative
4 **License Fees**	Renewal period	• **Within the last 2 months of each year** (Nov. 1 – Dec. 31)
	Grace period	• **2 months after** (Jan 1 – Feb. 28/29)
	LTO considered lapsed	• **After the grace period** (March 1); new license shall be applied for
5	Enforcing agency	• No clinical lab is allowed to operate w/out license from the Sec. of Health, thru the **Bureau of Research & Laboratories (BRL)**
6	Manage, administer or supervises a clinical lab	• **PHYSICIAN** Qualifications: ○ Licensed and duly registered w/ the Board of Medical Examiners of the RP, ○ Qualified in lab medicine & ○ Duly authorized by the Sec. of Health, authorization to be renewed ***annually***
7 **Definition of Terms**	• **Clinical laboratories**: (1) Clinical Pathology, (2) Anatomical Pathology, & (3) Forensic Pathology • **Pathology Assistant:** licensed physician with at least 1-year training in lab medicine; under the direct supervision of the head, or associate pathologist of the lab • **Laboratory Technician:** completed at least *2 yrs. of college* w/ at least *12 units* of lab subjects, & *1 year* of training in an accredited lab; w/out college work but at least *10 years* training & experience in a duly approved clinical Lab (PD 498)	
8 **Interpretation**	Authorized to interpret the provisions of AO 201	• Secretary of Health
9	Files the application for LTO	• Owner /authorized representative
	Approves LTO application	• Bureau of Research and Laboratories
10 **Terms and Conditions of License**	Not be assigned or transferred to unauthorized party (non-transferrable)	• License (LTO)
	Circumstances requiring clinical lab owner/rep to inform BRL	• Clinical lab desiring to transfer, change of ownership, administrator or manager or head – inform in writing the BRL *within 15 days after the transfer* has been completed
11 **Requirements for a Clinical Laboratory**	Minimum personnel	• Head (Pathologist) & • At least (1) medical technologist
	Ratio of clinical lab/BB managed by the head (pathologist)	• 1:4 (Pathologist: Clinical Lab/BB)
	Other Requirements	• Physical plants • Minimum Equipment & Apparatuses

12 **Reporting**	It serves as consultations between the requesting physician & the pathologists of the lab	• Laboratory Request
	It serves as a communication between the pathologists of the lab & the requesting physician	• Laboratory Results
	Necessitates reporting of results orally or in portion	• Emergencies situations upon the approval of the pathologist
13 **Recording**	How long should clinical lab records should be kept?	• At least 1 year
	How long anatomic & forensic pathology records should be kept?	• Permanently
16 **Exhibition of Certificate of Approval & License & the Rules & Regulations**	Location of CORs and LTO	• Conspicuous area
17 **Expiration**	Expiration of clinical lab LTO	• **Last day of the year** • December 31 (including the authorization to the Head)
18 **Renewal**	Renewal period	• **Last (2) months of each year** (Nov. 1 – Dec. 31)
	Grace period	• **(2) months after ED** (Jan 1 – Feb 28/29)
	LTO considered lapsed	• **After the grace period** (March 1) • During the pendency of the final action on their applications, unless otherwise advised or ordered by the Sec. of Health
19 **Inspection**	Person who conducts inspection	• **Secretary of Health or his representative**
	Gives authority to the Sec. of Health or his duly authorized rep/s the opportunity to inspect the clinical lab	• **LTO of the clinical laboratory**
21 **Modification and Revocation of Licenses**	License may be revoked, suspended or modified in full or part	• For any material false statement in the application and revealed during inspection
22 **Investigation of Charges or Complaint**	Conducts the investigation	• Secretary of Health or his duly authorized representatives

ADMINISTRATIVE ORDER No. 290 Series of 1976

AMENDING SECTION 11 SUBJECT A OR ADMINISTRATIVE ORDER NO. 201, SERIES OF 1973 CONCERNING REQUIREMENTS FOR CLINICAL LABORATORY

- Case: A registered medical technologist is working as a generalist in a hospital that caters 30-50 patients per day. On her way to the hospital for her night shift duty, she met a minor accident and immediately informed the head of the laboratory that she cannot report. Who will cover the duty for that night?
  - Physician-in-charge of the laboratory

SECTION	QUESTIONS	DETAILS
1 - Section II - **Requirements for a Clinical Laboratory**	Ratio of RMT to the number of tests/ examinations in the lab	• **1:800**
	Ratio of RMT to lab tech	• **1:2**
	Who shall cover the night duty in clinics & hospitals with <50 bed capacity?	• **Physician-in-charge of the lab**
	Number of clinical lab/BB managed by the head	• **1:3**
	Number of clinical lab managed by a licensed physician	• **1:1**
	• All MT & lab techs hired should be registered with Board of MT & should perform only the duties of the position for which they are hired	

ADMINISTRATIVE ORDER No. 49-B Series 1988

SECTION	QUESTIONS	DETAILS
1 **Purpose and Title**	***REVISED RULES AND REGULATIONS GOVERNING THE REGISTRATION, OPERATION AND MAINTENANCE OF CLINICAL LABORATORIES IN THE PHILIPPINES***	
4 **Scope**	• All entities performing the activities & functions of clinical labs which shall include the exam and analysis of any or all samples of ***human or animal***	
	Laboratories exempted from securing LTO	• Gov't hospital labs doing routine or min lab exams (services are extensions of gov't regional or central labs) • Labs used solely for academic training and or for purely research activities • *Physicians doing simple & routine exams* (routine hematology, urinalysis, fecalysis, & gram staining) for their own patients only in their own clinic
5 **Regulatory Authority**	Enforcing agency	• **Department of Health**, through the BRL in the office of Standards & Regulations
6 **Classification of Laboratories**	Classification of laboratory according to **function**	• Clinical pathology • Anatomic pathology • Forensic pathology
	Classification of laboratory according to **institutional character**	• Hospital labs or • Free-standing labs (non-hospital)
	Classification of laboratory according to **service extent and level**	• **PRIMARY** ○ Routine Hematology (CBC) ○ Routine Urinalysis ○ Routine Fecalysis ○ Gram Staining • **SECONDARY** ○ Tests in Primary ○ Routine hematology, UA, fecalysis, & gram staining ○ Routine Chemistry • **TERTIARY**
7 **Licensing**	Issued by	• **Bureau of Research & Laboratories (BRL)**
8 **Licensing Requirements (Technical Standards)**		• Staff • Physical Plant • Other Requirements
9 **Management of The Clinical Laboratory**	Pathologist as head is ***mandatory*** to these categories of clinical lab	• All categories of Free-standing clinical labs; • All **Tertiary** (3º) category hospital labs • All **Secondary** (2º) category hospital labs *(in areas w/ sufficient # of pathologist)*
	Pathologist as head is ***voluntary*** to these categories of clinical lab	• All **Primary** (1º) category hospital labs • All **Secondary** (2º) category of hospital labs *(in areas where pathologists are not sufficient)*
	Number of clin lab/BB can be managed by a pathologist	• **1:4**
	Number of hospital lab can be managed by physician with 3 months training	• **1:1**
10 **Quality Control Tests**	Authorized to conduct QC tests for the control of operations & as criteria for the renewal of licenses	• **Director of BRL**
11 **Reporting**		Results vs. request
12 **Recording**		
13 **Fees**		
14 **Basic License Requirements**		
15 **Application for License** – owner/rep; inspection (60 days)		
16 **License Fees**		
17 **Inspection**	Conducts inspection	• **Director of the BRL** or his duly authorized representative

18 **Issuance of License**	Persons who issues/approves the LTO	• Undersecretary for Standards & Regulations
	A license granted to hospital labs in areas where there are insufficient pathologists while the physician in charge is undergoing training	• Provisional License
	Validity of provisional license	• 1 year
19 **Terms and Conditions of License**	License is non-transferable	
	Circumstances requiring clinical lab owner/rep to inform BRL	• Clinical lab desiring to transfer, change of ownership, administrator or manager or head – inform in writing the BRL *within 15 days after the transfer* has been completed
21 **Expiration**	Expiry date of clinical lab LTO	• Last day of the year (December 31)
22 **Renewal**	Renewal period	• **1st 3 months of the year** (Jan 1 – March 31)
	Grace period	• **Next 2 months** (April 1 – May 31)
	LTO considered lapsed	• **After the grace period** (June 1) • During the pendency of the final action on their applications, unless otherwise advised or ordered by the Sec. of Health
23 **Violations**	• Operation of a clinical laboratory without a certified pathologist or without a qualified licensed physician or employing a person who is not an RMT • Change ownership, location, head of laboratory or personnel w/out informing the BRL • Refusal to allow inspection of the lab by the BRL • Refusal to perform tests quality controls by the BRL	
26 **Investigation of Charges or Complaint**	Authorize to conduct investigation	• **Undersecretary of Health for Standards & Regulations** or his duly authorized representative

ADMINISTRATIVE ORDER No. 2 s. 1998

- Amended administrative order No. 49-B s. 1998 Sections 16, 21, and 22 on License Fees, Expiration and Renewal of Clinical laboratory license

Section 16 License Fees	Section 21 Expiration	Section 22 Renewal
	• Based in the different regions starting from NCR (Mar)	• within 3 months before expiry date • Grace period – next (2) months after expiry date

ADMINISTRATIVE ORDER No. 59 s. 2001

SECTION	QUESTIONS	DETAILS
1 **Title**	**RULES AND REGULATIONS GOVERNING THE ESTABLISHMENT, OPERATION AND MAINTENANCE OF CLINICAL LABORATORIES IN THE PHILIPPINES**	
2 **Authority**	• DOH thru BHFS/CHD Health Regulation Cluster	
4 **Scope**	• Clinical labs which examine & analyze samples from ***humans***	
	Types of laboratories which are exempted from securing LTO	• Gov't hospital labs doing routine or min lab exams limited to ***acid fast bacilli microscopy, malaria screening & cervical cancer screening*** provided their services are declared as extensions of a licensed government clinical lab
5 **Classification of laboratories**	Classification of laboratory according to **function**	• Clinical Pathology • Anatomic Pathology – Forensic Pathology
	Classification of laboratory according to **Institutional Character**	• Hospital-based Laboratory • Non-hospital-based Laboratory
	Classification of laboratory according to **Service Capability**	• **PRIMARY** ○ Routine Hematology (CBC) ○ Routine UA ○ Routine Fecalysis ○ Blood typing- hospital based ○ Quantitative Platelet determination – hospital based • **SECONDARY** ○ Tests in Primary ○ Routine Hematology, UA, Fecalysis & gram staining ○ Routine Chemistry ○ Cross matching – hospital based • **TERTIARY** ○ Tests in secondary ○ Special chemistry ○ Special Hematology ○ Immunology/Serology ○ Microbiology
6 **Policies**	Approves permit to construct & design lay-out of a clinical lab	• BHFS
7 **Requirements & Procedures for Application of Permit to Construct and LTO**	Frequency of inspection	• Every 2 years or as necessary
	Number of days for BRL to be notified for a change in location	• Within 15 days before actual transfer
	LTO requirement for an extension laboratory	• Extension labs shall have a separate license
	Schedule of application for renewal of License	NCR – Jan-March 1,2,3 CAR – Feb-April 4,5,6 – March-May 7,8,9 – April-June 10,11,12, CARAGA, ARMM – May-July
	Grace period	• Next 2 months after expiry date with 1,000 penalty
	License issued by?	• BHFS/CHD
8 **Violations**	• Operations w/out a certified pathologist or w/out a RMT or employing a person not RMT • Change ownership, location, head of laboratory or personnel w/out informing the BRL • Refusal to allow inspection of the lab by the BRL; • Refusal to perform tests quality controls by the BRL • Gross negligence • Any act or omission detrimental to the public	
9 **Investigation of Charges or Complaint**	Person who conducts the investigation	• BHFS/CHD or his duly authorized representatives
10 **Modification & Revocation of License**		
11 **Repealing Clause**		
12 **Publication of List of Licensed Laboratories**		
13 **Effectivity**		

ADMINISTRATIVE ORDER (A.O) 2007-0027

QUESTIONS	DETAILS
REVISED RULES & REGULATIONS GOVERNING THE LICENSURE & REGULATION OF CLINICAL LABORATORIES IN THE PHILIPPINES	
Classification of laboratory according to **ownership**	• **Government**: operated & maintained, partially or wholly by the national LGU, any other political unit or any department
	• **Private**: owned, established & operated by any individual, corporation, association or organization
Classification of laboratory according to **function**	• **Clinical Pathology**: CC, Hema, Immunohema, Immuno, CM, Endocri, Mol.Bio, Cytogenetics, Toxicology & TDM
	• **Anatomic Pathology**: surgical, Immunohisto, cytology, autopsy, Forensic & Molecular Pathology
Classification of laboratory according to **institutional character** (AO 48 s. 2003)	• **Institution-Based**: lab that operated within the premises & as part of an institution, but not limited to hospital, medical clinic, school, medical facility, birthing home, psychiatric facility, drug rehabilitation center
	• **Freestanding**: a lab that does not form part of any other institution
Classification of laboratory by **Service Capability**	• **General Clinical Laboratory** A. Primary Category B. Secondary Category C. Tertiary Category D. Limited service capability • **Special Clinical Laboratory** - offers highly specialized lab services that are usually not provided by a general clinical lab

PRMARY CATEGORY	SECONDARY CATEGORY	TERTIARY CATEGORY
• Routine hematology • Qualitative platelet determination • Routine UA • Routine Fecalysis	• Services in primary plus: • Routine CC • Quantitative platelet determination • Cross matching – for hospital based • Gram Staining – for hospital based • KOH – for hospital based	• Services in secondary plus: • Special chemistry • Special hematology (coagulation) • Immunology • Microbiology

- **LIMITED SERVICE CAPABILITY** *(for institution based only)* – provides the lab tests required for a particular service in institutions such as but not limited to dialysis & social hygiene clinics

QUESTIONS	DETAILS

Clinical lab will only be granted LTO if:	• Clinical labs will comply with the standards and technical requirements formulated by the BHFS
	
	STAFFING:
	• Head: Pathologist
	• Licensed physician trained for *3 months* - may manage primary & secondary labs
	• Registered Medical Technologist (hospital based) – at least *1 RMT/shift*
	• Staff development thru training & appropriate continuing education
	• Additional: depends on workload: 10 mins/test
	  ○ Manual testing: 50 tests/RMT/8 hrs.
	  ○ Manual testing: 75 tests/RMT/12 hrs.
	  ○ Automated testing: 100 tests/RMT/8 hrs.
	  ○ Automated testing: 150 tests/RMT/12 hrs.
	
	PHYSICAL FACILITIES
	CL must:
	• Be well ventilated
	• Have enough working space
	• Have adequate water supply
	• Have these required measurements:
	  Category – Space in (sq.m.)
	  Primary – 10
	  Secondary – 20
	  Tertiary – 60
	
	CL WASTE MANAGEMENT
	• There shall be provisions for adequate and efficient disposal of waste following guidelines by the DOH
	
	QUALITY CONTROL PROGRAM
	• **Internal QC**: within the lab to monitor equipment, ensure competency and follow up on personnel, use/apply appropriate standard method. Reagents, supplies
	• **External QC**: designated by National Reference Laboratories or other recognized reference labs

Laboratories ***exempted from securing LTO*** but must be registered to BHFS	• Clinical labs that are operated & maintained exclusively for **research & teaching purposes**
	• **Special clinical labs** operating independent of a clinical lab *(Assisted Reproduction, Tech Labs, Molecular & Cellular Tech, Molecular Biology, Molecular Pathology, Forensic Pathology, Anatomic Pathology Labs)*
	• **NRL that is physically independent from the clinical lab of the hospital**
	  ○ Provided they are accredited by international accrediting/certifying body (CDC, WHO, & local accrediting/certifying body recognized by DOH)
	  ○ NRL designated by the DOH shall be covered by the license clinical lab of the hospital where they are respectively assigned

Physician's Office Laboratory (POL) will be REQUIRED to secure LTO if:	• Issue official lab results; • Perform more than monitoring examinations & • Cater not only physician's own patients
Department that supervises POCT in the hospital	• Pathology Department
It is a requirement for the referral of lab exams to outside labs	• **Memorandum of Agreement (MOA)** - serves as an *assurance of quality service* with a licensed clinical lab performing the services needed

LTO OF THE LABORATORY	
Requirements for the LTO of the Lab	• Issued in the name of *licensee & is non-transferable*; not valid for any premise/location other than what is stipulated in the application form
LTO validity	• **1 year** & **expires on the date stipulated** in the license unless suspended or revoked
Contents of the LTO of Non-Hospital based clinical lab	• Name of clinical lab • Name/s of owner or operator • Head of lab • Service capability • Period of validity • License # • Location of lab
	• LTO must be displayed in a ***prominent area***
Type of licensing of a hospital-based clinical lab	• ONE-STOP SHOP licensure & not required to obtain a separate license
	• Capability to perform HIV testing & or drinking water analysis shall be specifically indicated in the LTO
Requirement for a clinical lab & its satellite services w/in the same compound	• Shall have **1 LTO**
Requirement for a Satellite lab outside the central lab	• Shall require a **separate LTO**
Status of a mobile clinical lab	• Licensed as part of the *main clinical lab* & permitted to collect specimens only; allowed to operate only ***w/in 100km radius from the main lab***
Number of days for CHD to be notified for a change of headship	• w/in **2 weeks** from the initial date of implementation
RENEWAL OF LTO	
Renewal of HB clinical lab license	• Licensed in accordance to **One-stop-shop licensure system (OSS)**

Renewal of NHB clinical lab license	• **1st week of Oct-Nov. 30** of the current year w/ discount
	  ○ Oct. 1-Nov. 30 = 10% Discount
	  ○ Dec. 1-31 = No more Discount
	  ○ Surcharge Fee Starts Jan 1
	  ○ Freestanding/NHB = 50%
	  ○ HB under OSS = 100%

VIOLATIONS	
• Refusal of lab to participate EQAP by NRL or other external proficiency program approved by DOH; • Issuance of report, orally or in writing, in whole or portions which is not in accordance w/ the documented procedure approved by the head of the lab; • Permitting unauthorized persons to perform technical procedures; • Demonstrating incompetence or making consistent errors in the performance of clinical lab exams & procedures; • Deviation from the standard test procedures including use of expired reagents; • Reporting/releasing or erroneous results; • Lending or using name of licensed lab or head RMT to an unlicensed clinical Lab • Unauthorized use of the name & signature of the pathologist & RMT to secure LTO; • Reporting a test result for a clinical specimen even if the test was not actually performed; • Transferring of tests done in an outside lab to the result form of the referring lab • Performing & reporting tests in a specialty or subspecialty in which the lab is not licensed • Giving & receiving any commission, bonus, kickback or rebate or engaging any split-free arrangement in any form whatsoever w/any facility, physician, organization, agency or person, either directly or indirectly for patients referred to clinical lab	
Investigates & certifies if lab or personnel is accountable of alleged violation	• **BHFS/CHD Director** or representative/s
Authorize to suspend, cancel or revoke LTO	• **CHD Director** or representative/s
Penalty	• **Operation w/out LTO** – imprisonment (not <1 month) or fine of not <1,000 & not >5,000 or both • **If the offender is a firm or corporation** – managing head & or owners shall be liable

GUIDELINES ON POINT OF CARE TESTING IN HOSPITALS	
Rationale	• POCT permits testing to be performed at or near the patient's location & is a supplement to, not a replacement for the central lab services
Location of performing POCT	• At or near the patient's location
Role of POCT	• Supplement to, not a replacement for the central lab services
Persons who can perform POCT	• Lab personnel & non-lab staff, (clinical lab professionals are essentials to direct or oversee the overall program)
Responsible for the direction & supervision of testing by non-lab personnel	• Main Laboratory
What is POCT?	• Lab testing or near the site of patient care rather than in the clinical lab; it includes testing at the bedside, outpatient sites, w/in or outside the hospital or clinics, or at home
Objective of POCT	• Improve the time from the order of the test by the AP to the submission of the result to the physician (turn-around-time or TAT) • Clinically significant in the therapeutic TAT, i.e., the time from initiation of consultation to the time the appropriate critical treatment is given, (critical conditions, such as a MI or hypoglycemia in diabetic patients)
STAT TEST	• Test done on urgent cases, the results of which shall be released immediately, w/in *1 hr. after the procedure*. STAT is an abbreviation for *sta'tim* which means immediately
Benefits of POCT	• Improved survival rate of critical patients (shortened therapeutic TAT) • Immediate submission of the biochemical status of the patient leads to rapid progress in the management of the patient, earlier discharge either from the hospital or ER & a shorter length of stay • Used for screening & triaging or critical patients, preliminary diagnostic testing for emergency treatment & monitoring of status of patients under treatment (diabetics under insulin)
Hospital department in-charge pf the supervision of POCT	• Department of Pathology - designate a POCT Coordinator

POCT Coordinator	• Senior Member of the staff
	• Functions of POCT Coordinator:
	  ○ Recommends & maintains procedures for quality of the results of POCT
	  ○ Supervises the selection & maintenance of instruments, methods of testing, training of operators & quality control

COMMITTEE on POCT	
Its composition	• Representatives from the Dep't of Pathology, clinical dept's, nursing service & appropriate administrative dept's/units
Criteria for the selection of POCT	• Clinical indications for the performance of such tests at the bedside • Availability of such POCT tests from reliable suppliers • Substantially reduces therapeutic TAT; & • Cost-effectiveness of such POCT test
Areas in the hospital where POCT are located	• Critical Nursing Units – ICU, CCU, ER, Burn unit, OR, Recovery room, etc. • General Nursing Units – for monitoring of diabetics, electrolytes, oxygen saturation, coagulation status, etc. • Out-patient clinics & other units where POCT would be useful
Criteria for the selection of the brand/model of device/instrument for POCT	• From reliable supplier/distributor preferably w/ a good track record of service; • Give comparable results as main lab method • State of the art quality assurance features, including automatic calibrator • Portable and user-friendly (minimal steps to perform test); • Maintenance-free except for routine cleaning; & • May be connected to computer for data management and quality control
Referral of any problem related to POCT	• When the POCT device/instrument is not functioning well or the control sample is out of control range, the instrument should be referred to the Department of Pathology for resolution

Qualification & Training of Operators	• Medical Technologists or Physicians
	• Nurses or other paramedical personnel
	• Other hospital personnel maybe allowed to operate the POCT device/instrument provided that:
	  ○ Properly trained;
	  ○ Supervised by the staff of the Department of Pathology
	  ○ Performs quality control tests daily & submits results to the laboratory;
	  ○ Knows when to repeat tests when problems arise & when to refer problematic results to the Dep't of Pathology for resolution;
	  ○ Competency is checked periodically

Responsible for the supervision of operators of POCT	• POCT Coordinator from the Dep't of Pathology
	  ○ Reviews of QC results periodically (daily to weekly, depending on the # of tests)
	  ○ Checks the competency of the operators periodically
	  ○ Ensures that all the operators are properly trained

RECORDING & REPORTING OF POCT RESULTS	
Reports of POCT results	• POCT operator w/ the name of Head of the Laboratory or the POCT Coordinator
Legally responsible for the reports	• Pathologist
QUALITY CONTROL	
Frequency of running QC solutions by the POCT operator	• **At least once each day** and POCT devices/instruments are used or as recommended by the manufacturer
Reviews the QC results	• POCT coordinator
Frequency of the review of QC results	• **Daily or periodically**

- **Dep't Memorandum Order 2009-0086** - prescribes rules & regulations regarding the implementation of internal and external QC programs as requirements for the licensing of clinical laboratories as part of the Quality Assurance Program.
- **Dep't Memorandum Order 2009-0086-A** - requires clinical laboratory to participate in the National External Quality Assessment Scheme (NEQAS) administered by the designated NRLs

NATIONAL REFERENCE LABORATORIES (NRLs)

DO 393 s. 2000 - designates reference laboratories

Functions:

- Provide laboratory referral services - Confirmatory Testing, Surveillance, & Research
- Evaluate test kits and reagents, in coordination with the Bureau of Food & Drugs (BFAD), & Bureau of Health Devices & Technology (BHDT) of the DOH
- Maintain a Quality Assurance Program for Laboratory tests in coordination with the Bureau of Health Facilities and Services (BHFS) of the DOH
- Train Laboratory Personnel

NRL	SERVICES OFFERED
Research Institute for Tropical Medicine (RITM)	• NRL for Dengue, Influenza, TB & other mycobacteria, malaria, & other parasites, bacterial enteric diseases, measles & other exanthems, mycology, enteroviruses, anti-microbial resistance, & emerging diseases • NRL for confirmatory testing for Blood Donors & Blood Units
East Avenue Medical Center	• NRL for Environment & Occupational Health; Toxicology and Micronutrient Assay
National Kidney and Transplant Institute (NKTI)	• NRL for Hematology, Immunohematology, & Immunopathology
Lung Center of the Philippines (LCP)	• NRL for Anatomic Pathology and Biochemistry
San Lazaro Hospital - STD AIDS Cooperative Central Laboratory (NRL-SLH/SACCL)	• NRL for HIV-AIDS, Hepatitis, and STIs
Philippine Heart Center (PHC)	• Cardiac Markers

R.A. 1517
BLOOD BANK LAW

- Approved: **June 16, 1956**
- Aims to safeguard and promote public health and welfare by preventing improper collection, processing and sale of human blood or its product to the public.
- Repealed by RA 7719 (Approved on May 5, 1994)
- Implementing guidelines of the law are contained in AO# 9 s 1995, AO 17-A s 1998, and **AO 2008-0008**

SECTION 2

- Unlawful to establish, operate a blood bank (BB) or collect or process blood if he is not a licensed physician, or to sell blood collected from another person, without first securing a **license** from the DOH

SECTION 5

- ***Secretary of Health*** – responsible of strictly enforcing

AMMENDMENTS TO RA 1517

- AO 156 Series 1971 (1:3 [patho:BB])
- Letter of Ins. #42 (Blood trafficking & malpractice)
- AO 184 (PNRC & Chapters will perform the extraction & issuance of blood)
- AO 242 (Cost)
- AO 291 (1:2 [patho:BB]; 1:20 MT/extraction; 1:2 MT/technician)
- AO 293 (Cost)
- AO 133 (Memo 10: Medical Clearance)
- AO 116 (Cost/BT; honorarium fee)
- AO 57 (Classification of BB (ins. & category); 1:2 [patho:BB], 1:1 for physician w/ 3 months training)

ADMINISTRATIVE ORDER 156

Section 4	Renewal Period	• Within the last ***2 months of each year***
	Grace period	• Subject to a fine within the first 2 months of next year
	LTO considered lapsed	• After grace period and a new license shall be applied for
Section 5	It is a requirement prior to the operation of a BB	• No BB or Blood processing labs be allowed to operate w/out having secured proper license from the BRL properly signed by the Secretary of Health except in case of emergencies e.g. severe hemorrhage
	Signs the LTO	• Secretary of Health
Section 6	Required before any blood can be dispensed	• Proper prescription or Request
	Issues blood request	• Attending physician of hospital authority or another blood bank
Section 7	Requirements for BB manager or supervisor	• Licensed physician (Board Medical Examiner); & • Have training in BB operations (accredited BB institution)
Section 12		• The license is non-transferable
	Changes in the LTO that necessitates notification of the BRL	• Transfer/change of location • Termination/transfer of supervision • Cessation of operation
	Responsible proper notification of change/s	• BB owner of his representative
	Duration of the notification of BRL related to the changes	• 10 Days after the transfer
Section 13 **Requirement for BB & in Blood Processing Lab**	Number of blood banks a head can supervise (Head: BB)	• **1:3 or 1:1** (with outlets)
Section 14	Computation for the expiration date of blood	• **Expiration Date** = (Date of extraction + # of preservation days by anticoagulants used) – number of days in a month of extraction ○ Ex. (17 July + 35 CPDA1) – 31 ○ 52-31 = 21 Aug
Section 18	Expiration of LTO	• **December 31**

ADMINISTRATIVE ORDER 57

	Approved on	**January 3, 1999**
6 **Classification of BBs**	According to: • ***Service extent & level*** 1. Primary 2. Secondary 3. Tertiary • ***Institutional Character*** 1. Hospital BBs 2. Free standing (non-hospital)	
	Services Offered in each category	• **Primary Category:** ○ Donor Screening & Selection ○ Collection & processing of blood ○ Storage of Blood ○ Transport & issuances of blood ○ Compatibility testing (for hospital) ○ Transfusion of ***Whole blood*** (for hospital) • **Secondary Category** ○ Services Required for Primary Category ○ Provision of ***PRBCs, Plasma, & PRP*** ○ Investigations for transfusion (for hospital) • **Tertiary Category** ○ Services Required for Primary Category ○ Services Required of Secondary Category ○ Provision of ***Blood components, products or derivatives not included in Secondary Category*** ○ Investigation of incompatible crossmatches (for hospitals)
7 **License to Operate a BB**	It is a requirement prior to the operation of a BB	• **Proper & Valid License**
	Regulation body that issues LTO	• **BRL**
8 **Licensing Requirements for a BB (Technical Standards)**		
9 **Management of the BB**	Manager/ head of a BB	• Pathologist/Hematologist ○ Mandatory: for all categories of *freestanding* BBs & all *Secondary* & *Tertiary* categories of hospitals BBs ○ Voluntary: to all *Primary category* hospital BBs; may be headed by a licensed physician who has completed at least (3) months of training
	Number of BB a pathologist/ hematologist can manage	• **1:2**
	Number of BB a licensed physician with 3 months training can manage	• **1:1**
20 **Terms & Condition of License**	• Granted upon compliance w/ specific conditions & limitations in requirements • The license is non-transferable directly or indirectly to any authorized party	
	Changes in the LTO that requires notification of BRL	• Transfer/change of location: inform (15) days after transfer • Termination/transfer of supervision: inform (15) days after • Cessation of operation: notify in writing the BRL stating the date of termination
	Informs BRL of the change/s	• BB owner or his representative
	Number of days to notify the BRL of the changes	• 15 days after the transfer
22 **Expiration of License**	LTO Expiration date	• **December 31**
23 **Renewal**	Renewal period of LTO	• **First 3 months of the current year**
26 **Violations**		• Imposing charges on the blood products higher than those promulgated by BRL

RA 7719
NATIONAL BLOOD SERVICES ACT OF 1994

AMENDMENTS

- AO 9 s. 1995
- AO 4 s. 1996
- AO NO 17-A s. 1998
- AO 2008-0008

AO 9 s 1995

- Prescribes the principles, guidelines, procedures & standards for the implementation of RA 7719 (Voluntary Blood Donation Act)
- Approved: **April 28, 1995**

AO No. 17-A s. 1998

- It amends sections 26 to 32 of AO # 9 s 1995
- It is known as the Requirements and Procedures for a license to operate a blood banks/Center in the Philippines

AO 2008-0008

- One of the THRUSTS of the health sector reforms under FOURmula One (F1) for health – REGULATION
- To ensure access to quality & affordable services esp. to the poor.
- To provide public safe & adequate blood through efficient blood services network, thus separation of regulatory functions from programming functions will improve NVBSP

POLICIES & GUIDELINES		
CLASSIFICATIONS OF BSF	a. OWNERSHIP	1. **GOVERNMENT** – operated & maintained partially or wholly by a national, provincial, city or municipal gov't other political unit, by any dep't, division, board or agency or by gov't owned or controlled corp. 2. **PRIVATE** – (for hospital-based BSF only) – privately owned, established & operated with funds through donation, capital, or other means & by an individual, corp., association, or organization.
	b. INSTITUTIONAL CHARACTER	1. **HOSPITAL-BASED –** BSF located w/in the hospital premises 2. **NON-HOSPITAL-BASED –** gov't owned or PNRC owned BSF located outside the hospital premises
	c. SERVICE CAPABILITY	1. **BLOOD STATION (BS)** • Advocacy & promotion of voluntary blood donation & healthy lifestyle; • Provision of WB & PRBCs • Storage, issuance, transport & distribution of WB & PRBCs • Compatibility testing of red cell units, if hospital based. 2. **BLOOD COLLECTION UNIT (BCU)** • Advocacy & promotion of voluntary blood donation & healthy lifestyle; • Recruitment, retention & care of voluntary blood donors; • Screening & selection of voluntary blood donors • Conduct of health education & counseling services; • Collection of blood (mobile or facility-based) from qualified donors; • Transport of blood to BC for testing & processing; • Compatibility testing of red cells, if hospital based 3. **BCU/BS** – all services under BCU & BS 4. **BLOOD BANK (BB)** • Advocacy & promotion of voluntary blood donation & healthy lifestyle; • Storage & issuance of WB & blood components obtained from a BC; • Compatibility testing of red cell units; • Direct Coombs test; • Ted cell antibody screening • Investigation of transfusion reactions; • Assist the HBTC in the conduct of post-transfusion surveillance (hemovigilance) 5. **BLOOD CENTER (BC)** • Advocacy & promotion of voluntary blood donation & healthy lifestyle; • Recruitment, retention & care of voluntary blood donors; • Collection of blood (mobile or facility-based) from qualified voluntary blood donors • Conduct health education & counseling • Testing of units of blood for **Transfusion Transmitted Infections (TTIs);** • Processing & provision of blood components; • Storage, issuance, transport & distribution of units of WB and/or blood products to hospitals & other health facilities

LTO/ATO	Number of days CHD be notified	• 15 days - change in management, name or ownership • New LTO/ATO be applied – transfer or location
Inspection	Conducts inspection	• CHD Director or his representative
	Number of days for inspection after the receipt of application	• **Within 30 days after application**
RENEWAL OF LTO/ATO	Renewal of a non-hospital-based BSF	• **Every 3 years** (Oct. 1-Nov.30) on the last year of LTO validity period
	Renewal of a hospital-based BSF	• **Annually** (Oct.1-Nov.30) as part of the one-stop-shop license
	Renewal period	• Every 3 years on the last yr. of LTO validity period
VALIDITY OF LICENSE TO OPERATE	Validity period of the LTO of a non-hospital-based BSF	• Valid for 3 years • From Jan. 1 of the 1st yr. – Dec. 31 of the 3rd yr.
	Validity period of the LTO	• Valid for 1 yr. from Jan. 1-Dec.31 as part of hospital license
VIOLATIONS	a. Any material false statement in the application b. Misrepresentation of facts or falsification of documents or records. c. Refusal to make available its books, accounts & records of operation to person form BHFS/CHD d. Charging of service fees above maximum fees set by the DOH e. Collection from paid or remunerated donor f. Refusal to participate in EQAS	
PENAL PROVISIONS	Penalty for any BSF which collect charges and fees greater than prescribed	• License suspended or revoked by the CHD Director • Imprisonment of not <1 month nor >6 months, or fine not <P5,000.00 nor >P50,000.00, or both at the discretion of court
	Penalty of person who establish or operate BB w/out securing any license to operate from the Department or fails to comply with the standards prescribes	• Imprisonment of not <12 years nor >20 years of a fine of not (<P50,000 nor >500,000), or both at the discretion of the court
	Penalty of the head of the BSF & personnel responsible for dispensing or transfusing, incompletely tested &/or contaminated w/ blood or failure to dispose w/ in 48 hours blood contaminated w/TTIs after confirmatory from test	• Imprisoned for ten years (10)
	Performs confirmatory test	• Research Institute for Tropical Medicine (RITM)
		• CHD Director., after due notice & hearing, & upon the approval of Secretary, may impose the ff. administrative sanctions: ○ P5,000: penalty for BSF that fails to submit for renewal of LTO/ATO to CHD w/in 3 months before expiration ○ Recommendations to PRC to revoke the COR or invalidate license of any health professional

RA 7722
HIGHER EDUCATION ACT OF 1994

SECTION		DETAILS
1 **Title**	**May 18, 1994**	- **"Higher Education Act of 1994"**
3 **Creation of the Commission on Higher Education**	Referred to as the **Commission**	- **CHED** - referred to as the Commission; - Independent and separate from the (DECS - Department of Education, Culture, and Sports), & attached to the Office of the President for administrative purposes only
	Coverage of CHED	- Both public & private institutions of higher education as well as degree-granting programs in all post-secondary educational institutions, public & private
4 **Composition of the Commission**	Members of CHED	- **5 full-time members** - **Chairman of the Commission & (4) commissioners** - **Sec. of Education, Culture and Sports (DECS)** - ex officio chairman for a max period of (1) year; appointed by the President
	Qualifications of the Members	- Holders of earned doctorate (s), - Actively engaged in higher education for at least 10 years & - Not been candidates for elective positions in the elections immediately preceding their appointment
5 **Term of Office**	Appoints members of the commission	- **President -** appoints the full-time chairman and the commissioners for a term of 4 years, without prejudice to one reappointment
	Term of office of a full-time chairman	- **4 years**
	Term of office of the next (2) commissioners	- **3 years**
	Term of office of the last (2) commissioners	- **2 years**
8 **Powers and Function of the Commission**	Functions of CHED: - Formulate and recommend development plans, policies, priorities, and programs on higher education; - Formulate and recommend development plans, policies, priorities, and programs on research; - Recommend to the executive and legislative branches priorities and grants on higher education and research; - Set minimum standards for programs and institutions of higher learning recommended by panels of experts in the field and subject to public hearing, and enforced the same; - Monitor and evaluate the performance of programs and institutions of higher learning for appropriate incentives as well as the imposition of sanctions such as, but not limited to, diminution or withdrawal of subsidy, recommendation on the downgrading or withdrawal of accreditation, program termination or school course; - Identify, support and develop potential centers of excellence in program areas needed for the development of world-class scholarship, nation building and national development; - Recommend to the Department of Budget and Management (DBM) the budgets of public institutions of higher learning as well as general guidelines for the use of their income; - Rationalize programs and institutions of higher learning and set standards, policies and guidelines for the creation of new ones as well as the conversion or elevation of schools to institutions of higher learning, subject to budgetary limitations and the number of institutions of higher learning in the province or region where creation, conversion or elevation is sought to be made;	
12 **The Technical Panels**	Agency that reconstituted and/ or organized for different disciplines/ program areas by the commission.	
	Composition	- **Senior Specialists or academicians** from different disciplines or program areas
	Functions	- Assist the CHED in setting standards and in program and institution monitoring and evaluation

RA 8292
HIGHER EDUCATION ACT OF 1997

- Provides for the uniform composition and power of the governing boards of state universities & colleges (SUCs) nationwide

PROFESSIONAL REGULATION COMMISSION

3 FUNCTIONS OF THE PROFESSIONAL REGULATION COMMISSION	
QUASI-JUDICIAL	- **Investigation** of unlawful, inappropriate, and destructive behavior of examinees and professionals - The decision is as authoritative as that of a Regional Trial Court unless an appeal is made
QUASI-LEGISLATIVE	- **Formulation** of rules and policies that have the same binding effect as an actual law
EXECUTIVE	- **Implementation, administration, and enforcement** of regulatory policies of the national government with the maintenance of professional and occupational standards and ethics, and the enforcement of the rules and regulations

PD 223

- Creation of PRC
- Before PRC, the *Civil Service Commission* was in-charge of supervision and regulation of professional practice
- Amendments:
  - PD 657
  - RA 8981
- PRESIDENTIAL DECREE 223: "CREATING THE PROFESSIONAL REGULATION COMMISSION AND PRESCRIBING ITS POWERS AND FUNCTIONS"
  - Approved: June 22, 1973
  - 43 professions are under PRC

1 **Professional Regulation**		- a three-man commission to be known as the professional regulation commission (PRC) attached to the **Office of the President** for general direction & coordination **EO 565-A** - October 9, 2006 - Transferred by the president - PRC is attached to the Office of Department of Labor and Employment (DOLE) - In-charge: secretary of DOLE
2 **Composition**	Members of PRC and their corresponding term of office	- **1 full time commissioner** - head (**9 years**) - 2 full time members – Associate Commissioners; **6 years & 3 years** - Any vacancy shall be filled for the unexpired term only with the most senior of the associate commissioners succeeding the Commissioner at the expiration of his term, resignation or removal Board of PRC**:** - **Teofilo S. Pilando, Jr.** - Chairman - **Jose Y. Cueto, Jr.** - Commissioner - **Yolanda D. Reyes** - Commissioner - **Aristogerson T. Gesmundo** - Assistant Commissioner - **Architect Eric C. Nubla** - 1st Commissioner (Jan. 4, 1974) Board of Medical Technology: - **Dr. Marilyn A. Cabal-Barza -** Chairperson Member: - **Hon. Leila Lany M. Florento**
	Qualifications of the chairman or member of the Commission	- at least **40 years of age**, - familiar with the principles and methods of professional regulation and / or licensing and; - has at least (**5**) **years** of executive or managerial experience

5 Powers of the Commission	a) To administer, implement and enforce the regulatory policies of the National Government with respect to the regulation and licensing of the various professions and occupations under its jurisdiction including the maintenance of professional and occupational standards and ethics and the enforcement of the rules and regulations relative thereto. b) To perform any and all acts, enter into contracts, make such rules and regulations and issue such orders and other administrative issuances as may be necessary in the execution and implementation of its functions and the improvement of its services. c) To review, coordinate, integrate and approve the policies, resolutions, rules and regulations, orders or decisions promulgated by the various Boards with respect to the profession or occupation under their jurisdictions including the results of their licensures examination but their decisions on administrative cases shall be final and executory unless appealed to the Commission within thirty (30) days from the date of promulgation thereof. d) To administer and conduct the licensure examinations of the various Boards according to the rules and regulations promulgated by it; determine and fix the places and dates of examinations; appoint supervisors and room examiners from among the employees of the Government or private individuals who have been trained by the Commission for that purpose who shall be entitled to a daily allowance of not less than ten pesos (P10) for every examination day actually attended; use the buildings and facilities of public and private schools for examination purposes; and approve the release of examination results; e) To keep and maintain a register of authorized practitioners of the profession or occupation; issue certificates of registration or licenses signed by all the members of the Board concerned and the Commissioner with the official seal of the Board affixed; f) To have custody of all the records of the various Boards including their examination papers, minutes of deliberation, records of administrative cases and investigations and examination results; g) To determine, fix and collect the amount to be charged for examination, registration, registration without examination, licenses, annual registration fees, certifications, surcharges and other fees not specified under the provisions of Republic Act No. 6511 or amend the rates provided thereunder subject to approval by the Office of the President; h) To appoint, subject to the provisions of existing laws, such as officials and employees of the Commission as are necessary in the effective performance of its functions and responsibilities, prescribed their duties and fix their compensation; and to organize or reorganize the structure of the Commission, create or abolish positions, change the designations of existing positions to meet changing conditions or as the need therefor arises; Provided, That such changes shall not affect the employment status of the incumbents, reduce their ranks and/or salaries nor result separating them from the services; i) To submit and recommend to the President of the Philippines nominees for appointment as members of the various Boards from among those nominated by the bona fide professional organizations accredited by the Commission to fill existing or probable vacancies; j) The Commission may, upon the recommendation of the Board concerned, approve the registration of and authorize the issuance of a certificate of registration with or without examination to a foreigner who is registered under the laws of his country: Provided, That the requirement for the registration or licensing in said foreign state or country are substantially the same as those required and contemplated by the laws of the Philippines and that the laws of such foreign state or country allow the citizens of the Philippines to practice the profession on the same basis and grant the same privileges as the subject or citizens of such foreign state or country: Provided, finally, That the applicant shall submit competent and conclusive documentary evidence, confirmed by the Department of Foreign Affairs, showing that this country's existing laws permit citizens of the Philippines to practice the profession under the rules and regulations governing citizens thereof. The Commission is also hereby authorized to prescribe additional requirements or grant certain privileges to foreigners seeking registration in the Philippines if the same privileges are granted to or some additional requirements are required of citizens of the Philippines in acquiring the same certificates in his country;

6 Powers, functions and responsibilities of various boards	a) To look from time to time into the condition affecting the practice of the profession or occupation under their respective jurisdictions and whenever necessary, adopt such measures as may be deemed proper for the enhancement of the profession or occupation and/or the maintenance of high professional, ethical and technical standards and for this purpose the members of a Board may personally or through subordinate employees of the Commission conduct ocular inspection or visit industrial, mechanical, electrical or chemical plants or works, hospitals, clinics and other engineering works where registered practitioners of the profession or occupation are employed or are working for the purpose of determining compliance with the provisions of laws relative to the practice of the profession or occupation or as an aid in formulating policies relative thereto in accordance with the established policies promulgated by the Commission; b) To investigate violations of their respective laws and the rules and regulations promulgated thereunder and for this purpose may issue summons, subpoenas and subpoena duces tecum to alleged violators or witnesses thereof and compel their attendance to such investigations or hearings; c) To delegate the hearing of investigation of administrative cases filed before them except in cases where the issue involved strictly concerns the practice of the profession or occupation, in which case the hearing shall be presided by at least one member of the Board concerned assisted by a legal or hearing officer of the Commission; d) To promulgate decisions on such administrative cases subject to review by the Commission. If after thirty (30) days from the receipt of such decision no appeal is taken therefrom to the Commission, the same shall become final and immediately enforceable; e) Subject to review by the Commission to approve registration without examination and the issuance of the corresponding certificates of registration; f) After due process, to suspend, revoke, or reissue certificates of registration for causes provided for by law or by the rules and regulations promulgated therefor; g) To determine and prepare the contents of licensure examinations; score and rate the examination papers and submit the results thereof to the Commission within one hundred twenty (120) days after the last examination day unless extended by the Commission; and, subject to approval by the Commission determine the appropriate passing general rating if not provided for in the law regulating the profession.

Letter of Instruction 567

- **3 years**: expiration of license for all professionals

PD 657

- Date approved: **February 19, 1975**
- Amended Section 5 of paragraph (j) of PD 223:
  - The Commission may, upon recommendation of the Board concerned, approve the registration of and authorize the issuance of a certificate of registration with or without examination to a foreigner who is registered under the laws of his country: Provided, That the requirements for the registration or licensing in said foreign state or country are substantially the same as those required and contemplated by the laws of the Philippines and that the laws of such foreign state or country allow the citizens of the Philippines to practice the profession on the same basis and grant the same privileges as the subjects or citizens of such foreign state or country: Provided, further, That the applicant shall submit competent and conclusive documentary evidence, confirmed by the Department of Foreign Affairs, showing that his country's existing laws permit citizens of the Philippines to practice the profession under the rules and regulations governing citizens thereof: Provided, finally, That the Commission may, upon recommendation of the Board concerned, and approval of the President authorize the issuance of a certificate of registration without examination or a temporary special permit to practice the profession to any foreigner regardless of whether or not reciprocity exists in the practice of his profession between his country and the Philippines and under such conditions as may be determined by the Commission, if such foreigner is internationally known to be an outstanding expert in his chosen profession or a well-known specialist in any of its branches, and that his services are urgently necessary either for lack or inadequacy of local experts or if his services will promote the advancement of the profession in the Philippines. The Commission is also hereby authorized to prescribe additional requirements or grant certain privileges to foreigners seeking registration in the Philippines if the same privileges are granted to or same additional requirements are required of citizens of the Philippines in acquiring the same certificates in his country.

REPUBLIC ACT NO. 8981
PRC MODERNIZATION ACT OF 2000

1 **Title**	Date of approval?	- **December 5, 2000**
2 - **Statement of Policy**		
3 - **Professional Regulation Commission**		
4 **Composition**	Members of PRC & their term of office	- **1 full-time chairperson** - **2 full-time commissioners** - for **7 years** w/out reappointment
	Appoints the members of PRC	- Pres. of PH appoints commissioners
	Qualifications for the members of PRC	- at least **(40) years of age** - Holding a valid COR/professional license & a valid prof ID card or a valid certificate of competency or a valid prof license issued by any government agency, familiar with the principles & methods of professional regulation and/or licensing - at least **5 years of executive / management experience**; (1) of the Commissioners must be a past chairperson/member of a Prof Regulatory Board
7 **Powers, Functions and Responsibilities of the Commission**	- Require the completion of a refresher course (examinee failed 3 times); **approve the results of exams and release**; adopt measures to preserve the integrity and inviolability of licensure exams; appoint watchers; publish the list of successful examinees - BEQ: *Who approves and releases the results of exams?* Chairman of the Board - Issue COR or professional license; having the registrant's name, picture and registration number, signed by all the members of the board concerned and the chairperson. j) Upon recommendation of the Professional Regulatory Board concerned, to approve the registration of and authorize the issuance of a certificate of registration/license and professional identification card with or without examination to a foreigner who is registered under the laws of his state or country and whose certificate of registration issued therein has not been suspended or revoked: Provided, That, the requirements for the registration or licensing in said foreign state or country are substantially the same as those required and contemplated by the laws of the Philippines and that the laws of such foreign state or country allow the citizens of the Philippines to practice the profession on the same basis and grant the same privileges as those enjoyed by the subjects or citizens of such foreign state or country: Provided, further, That, the Commission may, upon recommendation of the Board concerned, authorize the issuance of a certificate of registration/license or a special temporary permit to foreign professionals who desire to practice their professions in the country under reciprocity and other international agreements; consultants in foreign-funded, joint venture or foreign-assisted projects of the government, employees of Philippine or foreign private firms or institutions pursuant to law, or health professionals engaged in humanitarian mission for a limited period of time: Provided, finally, That agencies, organizations or individuals whether public or private, who secure he services of a foreign professional authorized by law to practice in the Philippines for reasons aforementioned, shall be responsible for securing a special permit from the Professional Regulation Commission (PRC) and the Department of Labor and Employment (DOLE), pursuant to PRC and DOLE rules: k) To authorize any officer of the Commission to administer oaths: l) To supervise foreign nations who are authorized by existing laws to practice their professions either as holders of a certificate of registration and a professional identification card or a temporary special permit in the Philippines; to ensue that the terms and conditions for their practice or of their employment are strictly complied with; to require the hiring or employing government agency or private entity/institution to secure a temporary special permit from the concerned Board subject to approval by the Commission and to file a criminal complaint against the head of the government agency or officers of the said private entity/institution, who shall be liable under the penalty provided for in the concerned professional regulatory law or the penalty imposed pursuant to this Act, when the professional was hired and allowed to practice his/her profession without permit; to file upon due process request for deportation with the Bureau of Immigration and Deportation (BID); and to supervise professionals who were former citizens of the Philippines and who had been registered and issued a certificate of registration and a professional identification card prior to their naturalization as foreign citizens, who may, while in the country on a visit, sojourn or permanent residence, practice their profession: Provided, That, prior to the practice of their profession they shall have first been issued a special permit and updated professional identification card by the Board concerned subject to approval by the Commission and upon payment of the permit and annual registration fees; m) To monitor the performance of schools in licensure examinations and publish the results thereof in a newspaper of national circulation; n) To adopt and institute a comprehensive rating system for universities, colleges, and training institutes based on the passing ratio and overall performance of students in board examinations; o) To exercise administrative supervision over the various professional regulatory boards and its members; p) To adopt and promulgate such rules and regulations as may be necessary to effectively implement policies with respect to the regulation and practice of the professions; q) To implement the program for the full computerization of all licensure examinations given by the various professional regulatory boards including the registration of professionals not later than the year 2003 and other operations of the Commission; r) To investigate and decide administrative matters involving officers and employees under the jurisdiction of the Commission; s) To investigate motu proprio or upon the filing of a verified complaint, any member of the Professional Regulatory Boards for neglect of duty, incompetence, unprofessional, unethical, immoral or dishonorable conduct, commission of irregularities in the licensure examinations which taint or impugn the integrity and authenticity of the results of the said examinations and, if found guilty, to revoke or suspend their certificates of registration and professional licenses/identification cards and to recommend to the President of the Philippines their suspension or removal from office as the case may be; t) To issue summons, subpoena and subpoena duces tecum in connection with the investigation of cases against officials and employees of the Commission and the members of the Professional Regulatory Boards; u) To hold in contempt in erring party or person only upon application with a court of competent jurisdiction;	
8 **Regional Offices**	- Create regional offices as many as necessary	
9 **Powers, Functions, and Responsibilities of the Various PRB**	- Hear and investigate cases arising from violations, the rules and regulations promulgated and their codes of ethics, issue summons, subpoena and subpoena duces tecum: by decision of the PRB shall, unless appealed to the Commission, become final & executory after **(15) days (30-PD 223)** from receipt of notice of judgment or decision	
11 **Person to teach subjects for Licensure Exam on all Professions**	- Holders of valid COR and valid professional licenses of the profession and who comply with the other requirements of the CHED	

15 **Penalties for Manipulation & Other Corrupt Practices in the Conduct of Professional Examinations**	Penalty for a person who manipulates or rigs licensure exam results, secretly informs or makes known exam questions prior to the conduct of the exam or tampers with grades	- *Imprisonment of not <6 years & 1 day to not >12 years or a fine of not <P50,000 to not >P100,000 or both* at the discretion of the court
	Penalty if the offender is an officer or employee of the Commission or a member of the regulatory board	- Removed from office & perpetual absolute disqualification from public office in addition to the penalties prescribed
	Penalty for ***accomplice***	- Imprisonment 4 years & 1 day to 6 years or a fine P20,000 to not > P49,000 or both
	Penalty for ***accessories***	- Imprisonment 2 years & 1 day to 4 years or a fine P5,000 to not > P19,000 or both

Executive Order #200

- Institutionalized the partial computerization of the licensure examination
- CPA licensure exam were piloted
- Full implementation was made possible in 1993 (Test Databank & Optical Mark Reader)

PRC Resolution #11 s. 2011

- Prescribes guidelines for the registration of the Medical Laboratory Technicians

CONTINUING PROFESSIONAL DEVELOPMENT
Executive Order No. 226

- Institutionalized the continuing professional education (CPE) programs of the various professional regulatory boards (PRBs)
- CPE is the mandatory requirement for the renewal of a professional license
- Empowers the PRC to adopt policies on CPE programs that can enhance and maintain high professional, technical, and ethical standards in the practice of the profession for the implementation of the PRBs
- Assuring the rendition of highly qualitative professional service/s that will be globally competitive under the General Agreement on Trade Services (GATS) and securing the safety and protection of the public
- Effectivity: **July 5, 1995**

REPUBLIC ACT 10912
Continuing Professional Development Act of 2016

- *An Act Mandating and Strengthening the Continuing Professional Development (CPD) Program for all Regulated Profession, Creating the Continuing Professional Development Council, and Appropriating Funds thereof, and for Other Related Purposes*
- Lapsed into law last **July 21, 2016** after it was neither vetoed or signed by former President Benigno Aquino III
- CPD unit requirement is mandatory for all professionals before they can renew their PRC licenses
- Sponsored by Senator Antonio Trillanes III

PRC Resolution No. 2016-990

- In compliance with the new law, PRC issued amendments to the revised guidelines and the procedures of implementation of CPD programs
- Signed by chairman Teofilo S. Pilando, Jr. and commissioners Angeline T. Chua Chiaco and Yolana D. Reyes.
- Here are some of the amendments:
  - Each Professional Regulatory Board (PRB) shall create a **CPD Council** composed of *Chairperson and 2 members*
  - CPD Council accredits Local CPD Providers (Individual/Sole Proprietor, Firm/Partnership/Corporation, or Government Institutions/Agencies) and Foreign CPD Providers
  - Required CPD units of each profession must be earned every **three years**
- Matrix of required CPD units for each profession:
- 120 Credit Units: Accountancy
- **45 Credit Units**: Aeronautical Engineering, Agricultural Engineering, Agriculture, Architecture, Chemical Engineering, Chemistry, Civil Engineering, Criminology, Customs Broker, Dentistry, Electrical Engineering, Electronics Engineering, Environmental Planning, Fisheries, Forestry, Geodetic Engineering, Geology, Guidance and Counseling, Interior Design, Landscape Architecture, Librarian, Mechanical Engineering, ***Medical Technology***, Medicine, Metallurgical Engineering, Midwifery, Mining Engineering, Naval Architecture, Nursing, Nutrition and Dietetics, Occupational Therapy, Optometry, Pharmacy, Physical Therapy, Professional Teachers, Psychology, Radiologic Technology, Real Estate Service, Respiratory Therapy, Sanitary Engineering, Social Work, Veterinary Medicine
- 30 Credit Units: Chemical Technician, Dental Technician, Dental Hygienist, Master Electrician, Electronics Technician, Master Plumbing, Certified Plant Mechanic, Medical Laboratory Technician, Metallurgical Plant Foreman, Mine/Mill/Quarry Foreman, Detailman, X-Ray Technician, Real Estate Sales Person

PRC ID Renewal Period/Year - Minimum CPD CUs Required

- January-June 2017 - 0 unit
- July-December 2017 - 15 units
- 2018 - 30 units
- 2019 - 45 units
- 2020 onwards - 45 units

PHILIPPINE DANGEROUS DRUGS LAW

R.A. 6425	Title	- **Dangerous Drugs Act of 1972**
	Date of approval	- **April 04, 1972**
	Repealed by	- RA 9165
R.A. 9165	Title	- **Dangerous Drugs Act of 2002**
	Date of approval	- **June 07, 2002**
	Requirement prior to the operation of drug testing	- Drug testing laboratories must be licensed (ATO, LTO)
Categories of Drug Testing Center	1. Classification According to **Ownership**	- Government - Private
	2. Classification According to **Institutional Character**	- Institution-based - Freestanding
	3. Classification According to **Service Capability**	- **Screening Laboratory:** a laboratory capable of performing screening tests - Floor area: **20 square meters** - Working area: **10 square meters** - **Confirmatory Laboratory**: a laboratory capable of performing qualitative and quantitative examinations of dangerous drugs from the specimen - Floor area: **60 square meters** - Working area: **30 square meters**
BEQ: What do you call a person who will undergo drug testing? **Client/Donor**		
TECHNICAL REQUIREMENTS		
Physical Plant	Number of donors a drug testing lab can accommodate	- Whatever category which can receive/accommodate at least 5 prospective client/donors at a given time
	Other requirements	- Hand washing facility, - Toilet facility and - Stall for the orderly collection of specimen
Headship	Head of a drug testing lab	- **Pathologist**
	Number of drug testing labs a head may supervise	- May supervise 10 laboratory's provided that they are ***within 5 km radius*** -
	Type of laboratory a licensed physician with CL management training of DOH may supervise	- Screening laboratory
Reporting of Changes	Number of days' changes must be reported	- **15 days**
	Regulating Agency	- **CHD/BRL**
LTO	Validity	- Screening lab: **1 year** - Confirmatory lab: **2 years**
	Renewal	- **90 days before expiration**
	Lapsed	- **After 90 days**
Report	Person who signs the result form	- Analyst and Head - Specimen confirmed can be challenged **within 15 days**, not challenged **discard**; specimen kept **max of 1 year**

	Minimum information on the COC (Chain of Custody)	- Information identifying the specimen - Date and time of collection - Name of testing laboratory - Name and signature of all individuals who have custody of the sample during the collection process
Proficiency testing	Frequency of performing proficiency test	- **Thrice** (1 – repeat; 2 – revocation)
Sec. 36: **Authorized Drug Testing**	Regulating Agency	- **DOH**
	Authorized drug testing lab	- Any gov't forensic labs or by any accredited drug testing labs & monitored by the DOH
	Two testing methods	- **Screening**: determine the positive result as well as the type of the drug used - **Confirmatory**: confirm a positive screening test ***(gas chromatography/mass spectrometry)***
Sec. 37: **Issuance of False or Fraudulent Drug Test Results**	Penalties	- Issuance of false or fraudulent drug test results knowingly, willfully or through gross negligence: - Imprisonment (6) years & (1) day to (12) years & a fine (P100,000.00) to (P500,000.00) - Additional penalty through the revocation of the license to practice his/her profession in case of a practitioner, and - Closure of the drug testing center
Sec. 38: **Laboratory Examination or Test on Apprehended / Arrested Offenders**	Number of hours any person apprehended or arrested for violating the provisions of this Act shall be subjected to screening test	- **Within 24 hours**
	Number of days a specimen screened can be challenged Accrediting body	- Any person if positive for screening exam, test shall be challenged **within 15 days**; not challenged discard; specimens kept **maximum of 1 year**
Sec. 39: **Accreditation of Drug Testing Centers and Physicians**	Accrediting body	- DOH - Accredits both drug testing labs & physicians who shall conduct the drug dependency exam of a drug dependent
Violations	- Issuance of false/fraudulent results - Failure to protect the confidentiality of drug results - Failure to participate in proficiency testing - Conviction of the owner/manager of criminal offense committed as an incidence to the operation of the laboratory - Failure to refer (+) results to a confirmatory laboratory - Refusal to allow inspection/monitoring of BRL or CHD - Act contrary to the accepted clinical lab practice	

RANDOM DRUG TESTING	MANDATORY
1. Students of secondary and tertiary schools 2. Officers and employees of public and private offices	1. Applicants for driver's license 2. Applicants for firearm's license and for permit to carry firearms outside of residence 3. Officers and members of the military, police and other law enforcement agencies 4. All persons charged before the prosecutor's office with a criminal offense having an imposable penalty of imprisonment of not less than six (6) years and one (1) day 5. All candidates for public office whether appointed or elected both in the national or local government

RA 8504
"PHILIPPINE AIDS PREVENTION AND CONTROL ACT OF 1998"

- an act promulgating policies and prescribing measures for the prevention and control of HIV/AIDS in the Philippines, instituting a nationwide HIV/AIDS information and educational program, establishing a comprehensive HIV/AIDS monitoring system, strengthening the Philippine National AIDS Council

KEY FEATURES: PROVISIONS OF RA 8504	
ARTICLE 1: EDUCATION AND INFORMATION	
Sec. 4: **HIV/AIDS education in schools**	- DECS, CHED, and TESDA, utilizing official information provided by the DOH, shall integrate instruction on the causes, modes of transmission and ways of preventing HIV/AIDS and other sexually transmitted diseases in subjects taught in public and private schools at intermediate grades, secondary and tertiary levels, including non-formal and indigenous learning systems; *teachers will undergo seminar/training on HIV/AIDS prevention in coordination with DOH before they can teach the subject*
Sec. 5: **HIV/AIDS information as a health service**	- HIV/AIDS education and information dissemination shall form part of the delivery of health services by health practitioners, workers and personnel
Sec. 6: **HIV/AIDS education in the workplace**	- All government & private employees, workers, managers, and supervisors, including members of the (AFP) and (PNP), shall be provided with the standardized basic information and instruction on HIV/AIDS which shall include topics on confidentiality in the workplace and attitude towards infected employees and workers. - All government & private employees shall be provided w/ standardized basic information & instruction on HIV and AIDS. - HIV/AIDS education shall be integrated into orientation, training, continuing education, HR dev't programs, etc - Each employer shall develop, implement, evaluate and fund a workplace HIV and AIDS education and information program. - In collaboration with the DOH, DOLE shall oversee the anti-HIV/AIDS campaign in all private companies
Sec. 7: **HIV/AIDS education for Filipinos going abroad**	- HIV/AIDS education (seminar on the cause, prevention & consequences of HIV/AIDS) for Filipino workers before certification for overseas assignment
Sec. 8: **Information campaign for tourists and transients**	- Informational aids or materials on the cause, modes of transmission, prevention, and consequences of HIV infection shall be adequately provided at all international ports of entry
Sec. 9: **HIV/AIDS education in communities**	- LGUs, in collaboration with the DOH, shall conduct an educational and information campaign on HIV/AIDS
ARTICLE II: SAFE PRACTICES & PROCEDURES	
Sec. 12: **Requirement on the donation of blood, tissue, or organ**	- No laboratory or institution shall accept a donation of tissue or organ, unless a sample from the donor has been tested *negative for HIV*. All donated blood shall also be subjected to HIV testing and HIV (+) blood shall be disposed of properly and immediately. A second testing may be demanded as a *matter of right* by the blood, tissue, or organ recipient or his immediate relatives before transfusion or transplant, except during *emergency cases* - Testing of HIV (+): for *research purposes only* & *subject to strict sanitary disposal* requirements
Sec. 13: **Guidelines on surgical and similar procedures**	- DOH, in consultation and in coordination with concerned professional organizations and hospital associations, shall issue guidelines on precautions against HIV transmission during surgical, dental, embalming, tattooing or similar procedures - DOH shall likewise issue guidelines on the handling and disposition of cadavers, body fluids or wastes of persons known or believed to be HIV-positive; - Use of PPE

ARTICLE III: TESTING, SCREENING AND COUNSELING	
Sec. 15: **Consent as a requisite for HIV testing**	- No compulsory HIV testing. However, the State shall encourage voluntary testing for individuals with a high risk for contracting HIV - **Written informed consent** must first be obtained from the person concerned if he/she is of legal age or from the parents or legal guardian in the case of a minor or a mentally - Lawful consent to HIV testing be considered when: ○ a person volunteers or freely agrees to donate his/her blood, organ, or tissue for transfusion, transplantation, or research ○ a person has executed a legacy in accordance with Sec. 3 of RA 7170 (Organ Donation Act of 1991) ○ a donation is executed in accordance with Sec. 4 of RA 7170
Sec. 16: **Prohibition on Compulsory HIV Testing**	- **Compulsory HIV testing as a precondition** to employment, admission to educational institutions, the provision of medical service or any kind of service shall be deemed **unlawful**.
Sec. 17: **Exceptions to Prohibition on Compulsory HIV Testing**	- When charged with crime under the Revised Penal Code, Death penalty Act, Anti-rape Law - When the determination of the HIV status is necessary in resolving issues relevant to the Family Code - When complying with requirements of organ or blood donation
Sec. 18: **Anonymous HIV Testing**	- The State shall provide a mechanism for anonymous HIV testing and shall guarantee anonymity and medical confidentiality in the conduct of such tests
Sec. 19: **Accreditation of HIV Testing Centers**	- DOH - Accredits all testing centers, hospitals, clinics/labs performing HIV testing
Sec. 20: **Pre-test and Post-test Counseling**	- All testing centers, clinics, or laboratories which perform any HIV test shall be required to provide and conduct free pre-test counseling and post-test counseling for persons who avail of their HIV/AIDS testing services. - Counseling services must be provided only by persons who meet the standards set by the DOH
ARTICLE IV: HEALTH AND SUPPORT SERVICES	
- Right to health and the responsibility of state and society	
ARTICLE V: MONITORING	
- Monitoring (AIDSWATCH); recording of cases, contact tracing	
ARTICLE VI: CONFIDENTIALITY	
Section 30: **Medical Confidentiality**	- All health professionals, medical instructors, workers, employers, recruitment agencies, insurance companies, data encoders, and other custodians of any medical records, file, data or test results to observe strict confidentiality particularly the identity and status of persons with HIV
Section 31: **Exceptions to the mandate of Confidentiality**	- Medical confidentiality shall not be considered breached in the following cases: ○ when complying with reportorial requirements in conjunction with the AIDSWATCH programs provided in Sec. 27 of this Act, ○ when informing other health workers directly involved or about to be involved in the treatment or care of a person with HIV/AIDS; ○ when responding to legal proceedings where the main issue is the HIV status of a person
Section 32: **Release of HIV/AIDS test results**	- Result is released to: ○ Patient ○ Parent ○ Person authorized by AIDSWATCH Program ○ Legal guardian of insane person/orphan ○ Justice of the Court of Appeals or Supreme Court
Section 34: **Disclosure to Sexual Partners**	- Any person with HIV is obliged to disclose his/her HIV status and health condition to sexual partner at the earliest opportune time
ARTICLE VII: DISCRIMINATORY ACTS AND POLICIES	
Section 35: **Workplace Discrimination**	- Discrimination in any form from pre-employment to post-employment, including hiring, promotion or assignment, based on the actual, perceived or suspected HIV status of an individual is prohibited. - Termination from work on the sole basis of actual, perceived or suspected HIV status is deemed unlawful. - Persons w/ HIV/AIDS already employed by any public or private company shall be entitled to *the same employment rights, benefits and opportunities as other employees*. - HIV Infected employees shall act responsibly to protect own health & prevent HIV transmission - Acts of discrimination shall be reported to DOLE or to CSC

PENALTIES	
Sec 11: **Misleading information/advertising**	- 2 months to 2 years imprisonment
Sec 14: **Knowingly & negligently infecting others in the practice of one's profession**	- 6-12 years imprisonment, fines, suspension or revocation of license/accreditation
Sec 33: **Violations on medical confidentiality**	- 6 months to 4 years imprisonment, fines, suspension or revocation of license/accreditation
Sec 42: **Discriminatory acts & policies**	- 6 months to 4 years imprisonment, fines of not over P10,000, revocation of license/permits
ARTICLE VIII: MANDATE OF THE PHILIPPINE NATIONAL AIDS COUNCIL	
- To be the central advisory, planning and policy-making body for the comprehensive and integrated HIV/AIDS prevention and control program in the Philippines.	
ARTICLE IX: MISCELLANEOUS PROVISIONS	
Date of Approval	- **February 13, 1998**
BEQ: HIV "Window Period"	- refers to the period of time, usually lasting from **2 to 6 months** during which an infected individual will test "negative" upon HIV testing but can actually transmit the infection

RA 9288
"NEWBORN SCREENING ACT OF 2004"

Title	- "Newborn Screening Act of 2004"
Date of approval	- **April 07, 2004**
Purpose	- Shall ensure the following that: ○ every baby born in the Philippines is offered NBS; ○ the establishment and integration of a sustainable NBS System w/in the public health delivery system; ○ all health practitioners are aware of the benefits of NBS &of their responsibilities in offering it; & ○ all parents are aware of NBS and their responsibility in protecting their child from any of the disorders
Lead agency for the implementation	- **Department of Health (DOH)**
Sec. 5 **Obligation to Inform**	- Health practitioner who delivers, or assists in the delivery, of a newborn in the Philippines shall, prior to delivery, inform the parents or legal guardian of the newborn of the availability, nature and benefits of newborn screening
Sec. 6. **Performance of Newborn Screening**	- Performed **after 24 hours of life but not later than 3 days** from complete delivery of the newborn - Responsible in the performance of NBS: parent(s) and the practitioner or other person delivering the newborn
Sec. 7 **Refusal to be Tested**	- A parent or legal guardian may refuse testing on the grounds of religious beliefs, but shall acknowledge in writing their understanding that refusal for testing places their newborn at risk for undiagnosed heritable conditions - A copy of this **refusal documentation** shall be made part of the newborn's medical record and shall be indicated in the national newborn screening database
Sec. 9. **Licensing and Accreditation**	- DOH and the Philippine Health Insurance Corporation (PHIC)
Sec. 11. **Advisory Committee on Newborn Screening**	- Creation of Advisory Committee on Newborn Screening
	- 8 members: ○ DOH Secretary (Chairman); ○ Executive Director of the NIH (Vice Chairperson); ○ Undersecretary of the DILG; ○ Director of the Council for the Welfare of Children; ○ Director of the NBS Reference Center; and ○ (3) Representatives appointed by the DOH Secretary
Others	- Health facilities shall integrate NBS in its delivery of health services; - Creation of the Newborn Screening Reference Center at the National Institutes of Health and establishment and accreditation of NSCs equipped with a NBS laboratory and recall/follow up program

Disorder Screened	Effects Screened	Effects if Screened and Treated
Congenital Hypothyroidism	Severe Mental Retardation	Normal
Congenital adrenal hyperplasia	Death	Alive and normal
Galactosemia	Death, cataracts	Alive and normal
Phenylketonuria	Severe Mental Retardation	Normal
G6PD deficiency	Severe anemia, kernicterus	Normal

- **DOH Memo # 2012-0154 (May 15, 2012)**
  - Inclusion of ***Maple Syrup Urine Disease (MSUD)*** in the NBS panel
  - MSUD appears to be the most common inborn error of metabolism in the Philippines
  - Expanded screening will include 22 more disorders such as hemoglobinopathies and additional metabolic disorders, namely, organic acid, fatty acid oxidation, and amino acid disorders (Jan 2014)

PD 856
CODE OF SANITATION OF THE PHILIPPINES

Date of Approval	- **December 23, 1975**
Rationale	- Provides that the examination of drinking water should be performed only in water testing laboratories duly accredited by the DOH - Necessary to protect and safeguard the health and welfare of the community preventing the establishment, operation and maintenance of poorly equipped, substandard water testing laboratories
Personnel	a. **Head** ○ **Sanitary Engineer** (3 labs) ○ **Pathologist** (in areas without San. Engr.) b. **Technical Assistant** ○ **Chemist** (with 1-year experience as a chemist) ○ **RMT / Pharmacist** (3-years' experience in water analysis) c. **Lab Assistant** ○ **Lab aide** Personnel: Samples Bacteriology = 1:30 Routine Chemistry = 1:5
Conducts Inspection	- **BRL (Bureau of Research and Laboratory)**
Expiration of LTO & authorization	- **December 31st**
Renewal	- **Last 2 months of each year**

Sec. 11. **Types of water examination required**	
a. Initial examination	Physical, chemical & bacteriological exams & possible radioactive contaminant from newly constructed systems or sources before operation & opening for public use
b. **Periodic examination** (existing sources)	Bacteriologic: No longer than 6 months Chemical: Every 12 months / often Possible radioactive contamination: Yearly

Sec. 13 **Other Protective Measures**	
Distance of washing clothes or bathing area from well or other source for drinking	- More than 25 meter radius
Distance of artesians, deep or shallow well construction from any source of pollution	- More than 25 meters
Storage of radioactive sources or materials from well or drinking water source	- More than 25 meters; unless adequately and safely enclosed by proper shielding
Section 14. **Sanitary Permit**	- Requirement prior to operation of any food establishment - Changes must be notified within 14 days to the City or Municipal Health Officer
Section 15. **Health Certificates**	- Heath certificate as pre-employment requirement

AMENDMENTS

AO 31 s. 1979	- Prescribes the rules & regulations governing the ***accreditation of water analysis laboratories*** - Basic requirements: personnel, physical plant, equipment, apparatus, & other materials; manner of reporting the result; application for LTO; inspection; issuance & renewal of certificate to operate; violations; revocations of license; & updating requirements.
PD 1067	- **1976 National Water Code** - Date of effectivity: **December 31, 1976** - Designed to revise and consolidate the utilization, ownership, appropriation, development, exploitation, conservation, & exploitation of natural resources - Premise that all the waters in the Philippines belong to the state - Created the National Water Resources Council to control & regulate the water resources in the country
RA 9275	- **Philippine Water Act of 2004** - Provides consistent protection, preservation, & revival of the quality of water system in the Philippines - Main focus: water quality management system & international mechanisms & standards

REPUBLIC ACT 6969

- In 1990, the Phil. Congress enacted the ***Toxic Substances, Hazardous Wastes and Nuclear Waste Control Act of 1990***
- It seeks to protect the public health & environment from unreasonable risks posed by these substances.
- This legislation is under the jurisdiction of the **DENR with the Philippine Nuclear Research Institute (PNRI)** as the authority, specific for the control of nuclear/radioactive waste in the Philippines

Section 6. Function, Powers and Responsibilities of the Department of Environment and Natural Resources	Implementing Agency: Department of Environment and Natural Resources

TRAINING OF MEDICAL TECHNOLOGY INTERNS

AO 274 Series 1976

Title	• Rules and Regulations Governing Affiliation of Medical Technology Schools, Colleges and Universities in Government Hospitals under the Department of Health
Address to whom?	• Regional Directors, Chief of Agencies, and Training and Teaching Hospitals
Roles of laboratories	• Service: aid in diagnosis of disease for patient's management • Training of medical technology interns
Agency responsible in implementing the rules & regulations	• Department of Health (DOH)
I. Responsibilities of the DOH & Laboratories	1. Be licensed under RA 4688 2. Provide RMT in every section 3. Provide adequate lab space for staff & interns. 4. Provide sufficient equipment & supplies depending on the volume & type of examination; & 5. Accommodate at least 2 interns 6. Provide training program for MT interns Ratio of staff to interns = **1 : ≤ 10**
II. Responsibilities of the Affiliating Universities/ Schools/ Colleges of MT	1. Approval from the Department of Education & the Board of MT 2. Compliance w/ the provision of R.A. 5527 & PD No. 498 3. Provide an experienced laboratory instructor 4. Pay or replace (before) damage, breakage or losses by the MT intern 5. Ensure completion of theoretical aspects before internship. 6. Make a request for affiliation to the Chief of the Hospital 7. See to it that the laboratory instructor & students observe & abide the policies, rules & regulations of the hospital 8. See to it that the students & instructor are in their proper uniforms
III. Joint Responsibility of the Affiliating Universities/ Schools/ Colleges of MT & the Labs	1. Preparation and implementation of training program 2. Internship schedule 3. Monthly conference 4. Maintenance of harmonious relationship

AO. 21 Series 1979	RULES AND REGULATIONS FOR THE ACCREDITATION OF TRAINING LABORATORIES FOR MEDICAL TECHNOLOGY INTERNS	
	Date of Approval	• **April 09, 1979**
	Agency responsible for implementing the rules & regulations?	• Ministry of Health
	Ratio of staff to interns	• 1 : ≤ 10
	Changes in ATO	• Address, headship & other pertinent info- inform within 15 days after
	Inspection	• w/in 60 days
	Expiration	• 2 years after December 31
	Renewal	• 60 days before expiration

AO 5A	REVISED POLICIES, PROCEDURES AND GUIDELINES GOVERNING AFFILIATION AND TRAINING OF STUDENTS IN THE DEPARTMENT OF HEALTH (DOH) HOSPITALS & OTHER GOVERNING HEALTH FACILITIES	
	Date of approval?	**Feb. 17, 1996**
		NCATS: National Committee on Affiliation & Training of Students • Chairman: Director of Health Manpower Development and Training Service • Approval: Secretary of Health **RCATS: Regional Committee on Affiliation & Training of Students** • Chairman: Chief, HMDTD • Approval: Regional Health Director **HCATS: Hospital Committee on Affiliation & Training of Students** • Chairman/Approval: Chief of Hospital

AO No. 118-B s. 1992	RULES AND REGULATIONS GOVERNING THE ACCREDITATION OF CLINICAL LABORATORIES FOR TRAINING OF MEDICAL TECHNOLOGY INTERNS	
	Date of Approval	• **March 06, 1992**
	Inspection	• BRL (60 days)
	Expiration of LTO & authorization	• 2 years after date of approval
	Ratio of staff to interns	• **1 : ≤ 5**
	Renewal	• 60 days before
	Grace period	• 90 days after
	Lapsed	• After 90 days
	Issues/ revokes/ suspends LTO	• Undersecretary of Health for Standards & Regulations
	Regulatory agency	• Department of Health, through the BRL in the Office for Standards and Regulation
	Changes in LTO/ATO	• Address & others (15 days after)
	Basic requirements	• Same w/ lab except: valid LTO/BB training program & # of specimens/exams per section

A.O. 2007-0011
"RULES AND REGULATIONS GOVERNING THE ACCREDITATION OF CLINICAL LABORATORIES FOR TRAINING OF MEDICAL TECHNOLOGY INTERNS"

- BACKGROUND & RATIONALE
  - Health regulation under FOURmula One for Health
  - Assessment of 13 yrs. implementation of AO 118-B revealed issues on training standards, requirements on internship program & accreditation of laboratories
- Consultation meeting was convened & attended by representative from:
  - Board of Medical Technology of the PRC
  - Philippine Association of Schools of Medical Technology
  - PAMET
  - PSP
  - Selected gov't & privately owned Accredited Clinical Laboratories
  - Office of Programs & Standards of CHED
  - DOH offices, Health Human Resource Development Bureau Legal Service, & BHFS
- The participants resolved that the training standards & requirements for the medtech internship program is best handled by CHED. The resolution was formalized in MOA between DOH & CHED signed Dec. 2006

CMO # 14 s. 2006
POLICIES, STANDARDS & GUIDELINES FOR MEDICAL TECHNOLOGY EDUCATION

- OVERVIEW
  - It is in accordance to RA 7722 "Higher Education Act of 1994"
- To make Medical Technology Education at pace w/ the demands of GLOBAL competitiveness

ARTICLE 1: INTRODUCTION		
SECTION 1	Objective of Medical Technology Education/ Medical Science Education	• To develop a foundation in medical laboratory science & responsive in paramedical service w/highly innovative technologies to play its **VITAL ROLE** thus there must be combination of education, clin lab internship & specialized training
ARTICLE II: AUTHORITY TO OPERATE		
SECTION 2	It must be secured first by all private higher education institutions (PHEIs, State Universities & Colleges (SUCS) & local colleges & universities intending to offer BSMT/BSMLS program	• Authority from **CHED**
Article VIII: OTHER REQUIREMENTS		
Section 12 - **Program Administration**	Qualifications by a full-time dean/head of higher education institution offering BSMT/BMLS program:	• A Filipino citizen of good moral character; • Must be a RMT in the Philippines w/ at least master's degree in MT or other health related courses, Biological Science, Education & Administration; • Have **at least five (5) years** of very satisfactory teaching experience (MT professional subjects) & must have adequate managerial competence & technical expertise in school management.
Section 13 - **Faculty**	Qualifications of those teaching subjects in the licensure examination	• RMT with current & valid PRC license; • holder of a Master's degree in Medical Technology & other health related fields; • with a minimum of **one (1) year experience** in training of MT interns in an accredited laboratory; or in lieu thereof, **one (1) year experience** in a licensed clinical laboratory
	Qualifications of those teaching non-licensure Medical Technology subjects:	• a faculty member w/ appropriate Master's/Doctoral degree in related fields with at least two **(2) years teaching experience**
	Others	• In case of vacancy in the teaching force, a substitute or a replacement w/ similar or higher qualifications shall be employed. • Conditions of employment be in accordance w/ the institutional policy. The probationary period is in accordance w/ the Labor Law Code. Every college/university shall have a faculty manual containing info & policies pertaining to faculty. • Faculty members shall actively pursue continuing professional development. • Faculty members shall be actively involved in research activities. • Faculty members shall obtain a faculty evaluation rating of **"highly satisfactory."** • At least **fifty percent (50%)** of faculty handling professional subjects shall be in full time status

What is ETHICS?	
	• Ancient Greek word **"ethos" – "theory of things" or "way of living"** ○ Views the happening of events, decisions, and actions as the product of rightful thoughts and deeds

	• French word "*ethique*"; late Latin "*ethica*" - "of or for morals, moral, or expressing character", and modern Latin mores-"customs."
	  ○ ethics examines and investigates the justification of long-term moral judgments
	  ○ defines how individuals interact with another in a society with norms and values and shows what is good for the individual and society, and how established one's duty which one owes to oneself and to one another
	  ○ covers the concepts and principles of defending, systematizing, remodeling, and Interconnecting the implications of right and wrong deeds

Ethics as a Philosophy	• Touches the principles of righteousness and goodness; includes the so-called code of right conduct (code of ethics) in the everyday struggle for life
Ethics as a Science	• aimed at attaining a systematized knowledge and experience • science of the highest goodness because it investigates what is right or good in human conduct and prescribes what humans ought to do in terms of obligations, rights, specific virtues, benefits to society, and fairness
Ethics vs Morality	• Ethics: theory of good or evil and right or wrong; refer to what has been known as the standard behavior or conduct in a particular place • Morality: connotes the existence of real-world beliefs and practices about proper human conduct; refers to any behavior or practice common to all groups
General Types of Ethics	• **Meta-ethics**: ethics far beyond the physical world, that is, the rule of power and the will of Almighty God over human reasoning
	• **Applied ethics**: branch of ethics that refers to the analysis of moral issues such as abortion, animal rights, and euthanasia
	• **Normative ethics**: implies the institution of moral conduct in regulating what is right or wrong. The basic principle following normative ethics is the Golden Rule, which is, however, governed by three theories: virtue theory, duty theory, and consequentialist theory
Approaches to ethics	• **Virtue approach**: stems from the description of moral behavior by Plato and Aristotle as "what a moral or virtuous person does."
	• **Deontological "duty":** "deontology" is derived from the greek word deon(t) which simply means "bind." Denotes duty on the part of the agent or doer of the action. Obligation is given more importance rather than the outcome of the act.
	• **Consequentialist** (teleological) ethics: based on the premise that if the outcomes are good, then the related actions can be good regardless of the motive of the actor.
Professional Ethics	• applies to the workforce and professionals. Professionals act according to certain standards of behavior
Principles of Ethics	• Every individual acting in its own capacity usually carries the burden of ethical responsibility. Professional ethics emanates from the moral reflection of professional endeavors enveloped in a code of ethics.
Ethical behavior RMTS are expected to demonstrate and manifest	• Impartiality and Objectivity • Openness and Full Disclosure • Due Diligence • Confidentiality • Fidelity to professional responsibilities • Reliability at work • Avoidance of conflict of interest

PRINCIPLE AND RULES OF THE CODE OF ETHICS OF THE MEDICAL TECHNOLOGY PROFESSION	
Principle of Code of Ethics 1	• An RMT must accept the responsibility of upholding the laws in line with the profession. • **Rule 1.1.** Must render laboratory services competently. • **Rule 1.2.** Must engage in only those aspects of the profession that are within the scope of his/her professional practice and competence, considering his/her level of education, training, and experience. • **Rule 1.3.** Must not defraud or engage in any scheme to defraud in connection with the obtaining payment, reimbursement, or grants for services rendered, research conducted, or products dispensed.
Principle of Code of Ethics 2	• An RMT must work harmoniously with other health workers. • **Rule 2.1.** Must fervently take his/her responsibility to achieve and maintain the highest level of professional competence and performance. • **Rule 2.2.** Must uphold the dignity and autonomy of the profession, maintain harmonious intra-professional and inter-professional relationships, and accept the professions' self-imposed standards.
Principle of Code of Ethics 3	• An RMT must adapt the ethical value of fairness with fellow medical technologists in the delivery of laboratory services. • **Rule 3.1.** Must not participate in professional activities that constitute a conflict of interest. • **Rule 3.2.** Must refer those served professionally solely on the basis of interest of those being referred and not on any personal interest, financial or otherwise. • **Rule 3.3.** Must engage in one employment at a time.
Principle of Code of Ethics 4	• An RMT must have full confidence in showing competency in laboratory work. • **Rule 4.1** Must use every resource, including referral when appropriate, to ensure that high quality service is provided. • **Rule 4.2** Must discriminate in the delivery of professional services or the conduct of research and scholarly activities on the basis of race or ethnicity, gender, gender identity/gender expression, age, religion, national origin, sexual orientation, or disability.
Principle of Code of Ethics 5	• An RMT must share knowledge and contribution to professional organizations and in the field of scientific research. • **Rule 5.1.** Must acknowledge the source when using other persons' ideas, research, presentations, or products in written, oral, or any other media presentation or summary.
Principle of Code of Ethics 6	• An RMT must honor his/her responsibilities to the professions and relationship with colleagues, students, and members of other professions and disciplines • **Rule 6.1** Must not discriminate in his/her relationship with colleagues, students, and members of other professions and disciplines on the basis of race or ethnicity, gender, gender identity/gender expression, age, religion, national origin, sexual orientation, or disability. • **Rule 6.2** A medical technologist's statements to the public when advertising, announcing, and marketing his/her professional services, reporting research results, and promoting products shall adhere to professional standards and shall not contain misrepresentations.
Principle of Code of Ethics 7	• An RMT must take his/her responsibility to hold paramount the welfare of persons he/she serves professionally or who are participants in research and scholarly activities, and he/she shall treat animals involved in research in a humane manner. • **Rule 7.1** Must not misrepresent his/her credentials, competence, education, training. experience, or scholarly or research contributions. • **Rule 7.2** Must fully inform the persons he/she serves of the nature and possible effects of services rendered and product dispensed, and shall inform participation research about the possible effects of their research participation in research conducted.
Principle of Code of Ethics 8	• An RMT must honor the dignity of the profession and professional organization. • **Rule 8.1** Must not reveal, without authorization, any professional or personal information about identified persons served professionally or identified participants involved in research and scholarly activities unless doing so is necessary to protect the welfare of the person or of the community or is otherwise required by law.
Principle of Code of Ethics 9	• A registered medical technologist must maintain professionalism all the time. • **Rule 9.1** Must honor his/her responsibility to achieve and maintain the highest level of professional competence and performance.
Bioethics vs Medical ethics	• **Bioethics**: a combination of ethical theory and various disciplines such as medicine, law, social science, philosophy and the like to address the ethical issues of clinical decision
	• **Medical ethics**: deals with moral values and decisions as they are applied to medicine.

PRINCIPLES OF BIOETHICS	
Respect for Autonomy	• Respect for autonomy is the act of respecting the decisions of others. A person should control his/her own life. • Prioritization of one's decision. It is always the decision of the patient that prevails since such a decision is more beneficial rather than harmful. • Intervention from other people. is adopted when the patient is terminally ill such that the physician should decide on the best treatment the patient should have.
Beneficence	• Principle of doing an action that benefits others. Doing good things solves ethical problems and dilemmas. The propositions coming from the act of benevolence: (1) prevent the infliction of needless pain; (2) prevent killing a person; and (3) prevent incapacitating the others.
Non-maleficence	• The avoidance of doing "harm" to others. Some propositions associated include the following: (1) do not kill; (2) do not cause needless pain; and (3) do not incapacitate others.
Justice	• Holds that ethical theories, especially legal ethics, should impose fairness to everyone involved. It holds that clinical decisions should be consistent with ethical theories. • A clinical decision that contains justice should have a logical and legal basis relative to the decision to be made.

Rights of Healthcare Providers	1. Be treated with respect as a person 2. Be involved in whatever care they need 3. Have a written agreement covering the services the healthcare providers have agreed upon 4. Have the care and services be reviewed 5. Have privacy and confidentiality of personal information 6. Have a compensation that is just and fair, transparent, and accessible 7. Have equal access to due process and opportunities
Responsibilities of Healthcare Providers	1. Respect the rights of patients 2. Solicit informed consent from the patients before treatment and examination 3. Provide basic first aid in case of emergency as dictated by duty 4. Attend to the needs of patients depending on the authority given 5. Provide a seamless continuum of care 6. Deliver services effectively and efficiently without compromising quality 7. Spend public funds prudently and wisely 8. Provide a clear policy about healthcare professionals reporting concerns about risks for patients 9. Provide a safe working environment 10. Respect healthcare personnel 11. Ensure wheelchair accessibility 12. Ensure that continuous care is available 13. Be within reach, or on stand-by in a specific place 14. Provide on-duty services 15. Render services within the required number of hours
Duties and Responsibilities of Patients	1. Patients should communicate openly to healthcare providers. 2. Patients should provide their complete medical history. 3. Patients and physicians should mutually agree on the treatment and goals of therapy. 4. Patients should pay the physicians and healthcare providers with just compensation for services rendered. 5. Patients should be committed to health maintenance by demonstrating health-enhancing behavior 6. Patients should show concern for other people. 7. Patients should participate in medical education.
Rights of Patients	1. The right to information. 2. The right to choose medical care providers. 3. The right to self-determination. 4. The right to religious belief. 5. The right to medical records. 6. The right to leave hospital premises. 7. The right to refuse to be subject of medical research. 8. The right to express grievances,
Cardinal Virtues of Healthcare Providers	• Prudence • Justice • Fortitude • Temperance

CODE OF ETHICS

- Ethics is defined as principles of product governing an individual or a group.
- CODE OF ETHICS OF MEDICAL TECHNOLOGISTS
  - ensure proper behavior leading to mutual respect among medical technologists and among other members of the health care team. These code of ethics are read during oath-taking ceremonies of newly-licensed MT

BOARD OF MEDICAL TECHNOLOGY CODE OF ETHICS

As I enter into the practice of Medical Technology, I shall:

- Accept the responsibilities inherent to being a professional.
- Uphold the law and shall not engage in illegal work nor cooperate with anyone engaged.
- Avoid associating or being identified with any enterprise of questionable character
- Work and act in a strict spirit of fairness to employer, clients, contractors, employees and in a spirit of personal helpfulness and fraternity toward other members of the profession.
- Use only honorable means of competition for profession employment or services and shall refrain from unfairly injuring, directly or indirectly the professional reputation, projects or business of a fellow medical technologist.
- Accept employment from more than one employee only when there is no conflict of interest.
- Perform professional work in a manner that merits full confidence and trust with absolute reliability, accuracy, fairness and honesty.
- Review the professional work of other medical technologists when requested, fairly and in confidence whether they are subordinates or employees, authors of proposals for grants or contracts, authors of technical papers or other publications or involved in litigation.
- Advance the profession by exchanging general information and experience with fellow medical technologists and other professionals and by contributing to work of professional organizations.
- Restrict my praises, criticisms, views and opinions within constructive limits and shall not use the knowledge known for selfish ends.
- Treat any information I acquired about individuals in the course of my work as strictly confidential and may be divulged only to authorized persons on entities or with consent of the individual when necessary.
- Be dedicated to the use of clinical laboratory science to promote life and benefit mankind.
- Report any infractions of these principles of professional conduct to the authorities responsible for enforcement of applicable laws or regulations, or to the ethics committee of the organization.
- To these principles, I hereby subscribe and pledge to conduct myself at all times in a manner of befitting the dignity of my profession.

OATH OF PROFESSIONAL

I, (name) of (address) hereby solemnly swear, that will support and defend the Constitution of the Philippines; that I will bear true faith and allegiance to the same, that will obey the laws, legal orders, and decrees promulgated by the duly constituted authorities of the Republic of the Philippines; and that impose this obligation upon myself voluntarily, without mental reservation or purpose of evasion.

I further solemnly swear, that at all times and places, I will adhere closely to the Ethical and Professional Rules generally accepted by the MEDICAL TECHNOLOGY Profession in the Philippines, and I will well and faithfully discharge, to the best of my ability, the duties and obligations, incumbent upon a legally authorized MEDICAL TECHNOLOGY Practitioner.

So help me GOD.

PANUNUMPA NG PROPESYONAL

Ako, si (pangalan) ng (Pook na Sinilangan, Bayan/Lungsod, Probinsya) ay taimtim na nanunumpa na itataguyod ko at ipagtatanggol ang Saligang Batas ng Pilipinas, na ako ay tunay na mananalig at tatalima rito; na susundin ko ang mga batas, mga utos na legal, at mga atas na ipinahayag ng mga sadyang itinakdang may kapangyarihan ng Republika ng Pilipinas; at kusa kong babalikatin ang pananagutang ito, na walang ano mang pasubali o hangaring umiwas.

Taimtim pa rin akong manunumpa na sa lahat ng panahon at pook na kinaroroonan ay mahigpit akong manghahawakan sa mga etikal at tuntuning propesyonal ng mga Medikal Technolohist sa Pilipinas, at marapat kong gagampanan ng buong husay sa abot ng aking makakaya ang mga tungkulin at pananagutang iniatang sa isang itinakdang Medikal Technolohist. (Propesyon)

Kasihan Nawa ako ng Diyos.

MEDICAL TECHNOLOGIST'S PRAYER

God, who by calling us to the vocation of a medical technologists, has placed upon us the obligation of being a constant help in the scientific care of the sick, grant us by thy divine light a deep insight into the serious responsibilities of our task

By thy divine wisdom, awaken in us a growing zeal and determination to increase our knowledge of how to search for the underlying causes of sickness and disease; how to recognize the evidence of physical changes; how to make important chemical analyses, and other valuable test so helpful in caring for the sick

By thy divine love, permit us in this way to share with those who directly care for the sick, that thus we may be of constantly working through the eternal physician, Christ our Lord, Amen.

SUMMARY OF MEDTECH RELATED LAWS		
RELATED LAWS	**TITLE AND DESCRIPTION**	**DATE OF APPROVAL**
RA 6425	Dangerous Drugs Act of 1972	April 4, 1972
RA 9165	Comprehensive Dangerous Drugs Act of 2002	June 7, 2002
PD 893	Reconstituting the National Schistosomiasis Control Commission into the Schistosomiasis Control Council and for Other Purposes	February 16, 1976
PD 996	Providing for Compulsory Basic Immunization for infants and Children Below Eight Years of Age	September 16, 1976
AO 55 s. 2000	Guidelines in the implementation of MASS Treatment Strategy for Schistosomiasis Control/Elimination	May 30, 2000
Republic Act No. 8749	Philippine Clean Air Act of 2004	July 17, 2003
AO 81, S. 2003	Rules and regulations governing the Accreditation of Hospitals Engaged in the conduct of kidney transplantation	September 1, 2003
RA 9275	Philippine Clean Water Act of 2004	March 22, 2004
AO 2005-0002	Rules and regulations for the establishment of the Philippine National Blood Services Amending Pertinent Provisions of Administrative Order No. 9, Series of 1995 (Rules and Regulations Implementing R.A. 7719 otherwise known as the National Blood Services Act of 1994)	January 10, 2005
CMO 14, s. 2006	Policies Standards and Guidelines for Medical Technology Education	March 15, 2006
AO 2006-0024	Rules and regulations governing the accreditation of laboratories for drinking water analysis	May 10, 2006
AO 2006-0015	Implementing Guidelines on Hepatitis B immunization for Infants	June 23, 2006
DOH AO 2006-0026	Implementing Guidelines in the Conduct of the National TB Control Program Directly Observed Therapy Short-Course (NTP-DOTS)	July 4, 2006
AO 2007-0014	Guidelines on the issuance of Certificate of Product Registration for Equipment or Devices Used for Treating Sharps, Pathological and Infectious Waste	March 14, 2007
AD 2007-0011	Repeal of A.O. 118-8, s. 1992, Entitled "Rules and regulations governing the accreditation of clinical laboratories for the training of medical technology interns	April 4, 2007
CMO 6, s. 2008	Guidelines for the Accreditation of Clinical Laboratories involved in the Training of Medical Laboratory Science/Medical Technology Interns	March 31, 2008
AO 2007-0019	Guidelines for the implementation of the quality assurance system on Direct Sputum Smear Microscopy (DSSM)	April 11, 2007
AO 2007-0005	Revised Guidelines in the Management and Prevention of Schistomiasis	May 7, 2007
RA 9482	Anti-Rabies Act of 2007	May 25, 2007
EO 663	Implementing the National Commitment for "Bakuna ang Una sa Sanggol at Ina"	Sept. 21, 2007
AO 2007-0027-A	Amended certain provisions of AO 2007-0027 Exempted Nat'l TB Reference Laboratories(NTRL) & Regional TB Reference Laboratories performing TB culture	August 28, 2008
AO 2007-0028	Attaining the WHO's Goal to Eliminate Measles and Neonatal Tetanus, Eradicate Polio, Control Hep. 8 and other Vaccine Preventable Disease	October 2, 2007
AO 2008-0002	Posting of Schedule of Fees for Laboratory and other Diagnostic Services of Hospitals and other Health Facilities	January 15, 2008
AO 2008-0004	Revised National Policy on Kidney Transplantation from Living Non related Organ Donor and its Implementing Structures	March 3, 2008
AO 2008-0004A	Amendment to AO 2008-0004 on Revised National Policy on Kidney Transplantation from Living Non-related Organ Donor and its Implementing Structures	May 29, 2008
AO 2008-0007	Schedule of Fees for the Licensure of General Clinical Laboratories and the Registration of Special Clinical Laboratories	March 14, 2008
AO 2008-0011	Revised Guidelines for Implementing Tuberculosis Control Program in Children	May 21, 2008
AO 2008-0018	Guidelines for Implementation of the Programmatic Management of Drug Resistant Tuberculosis (PMDT)	May 26, 2008
AO 2008-0022	Policies and Guidelines in the Collaborative Approach of TB and HIV Prevention and Control	August 8, 2008
AO 2008-0025	Guidelines on implementation of the Integrated Drug Test Operations and Management information System (IDTOMIS) for Screening and Confirmatory Drug Test Laboratory Operation	July 29, 2008
AO 2009-0001	Revised Policy and Guidelines on the Diagnosis and Treatment of Malaria	January 13, 2009
AO 2009-0003	Technical Guidelines for Implementing DOTS Strategy in Jails and Prisons	January 8, 2009
AO 2009-0006	Guidelines on Antiretroviral Therapy (ART) among Adults and Adolescents with Human Immunodeficiency Virus (HIV)	January 13, 2009
AO 2009-0009	Application of Department of Health Policies, Plans and Guidelines Pertaining to Influenza A (H1N1) Virus	May 4, 2009
AO 2009-0013	Declaring the Month of July Every Year as the Mass Treatment and Awareness Month for Schistosomiasis in the Established Endemic Areas in the Philippines	April 14, 2009
AO 2009-0014	Guidelines for the DOH Integrated Drug Test Operations and Management Information System (IDTOMIS) Subscription/User Fee Collection and Utilization	May 9, 2009
AO 2009-0016	Policies and Guidelines on the Prevention of Mother to Child Transmission (PMTCT) of Human Immunodeficiency Virus (HIV)	May 20, 2009
AO 2009-0020	Interim Guidelines for the Disposal of Health/Medical Records of Private Healthcare Facilities Damaged by Typhoon	October 22, 2009
AO 2009-0021	Diagnosis and Treatment Guidelines for Capillariasis Infection	November 16, 2009
AD 2010-0001	Policies and Guidelines for the Philippine National Blood Services (PNBS) and the Blood Services Network (BSN)	January 6, 2010
AO 2010-0002	Policies and Guidelines Pertinent to the Establishment and Operation of Local Blood Councils to Support the Implementation of the National Voluntary Blood Services Program for Blood Safety and Adequacy, Quality Care and Patient Safety	January 7, 2010
RA 10069	Health Workers' Day Act- An act declaring May 7 of every year as "Health Workers Day"	April 6, 2010
AO 2010-0009	Guidelines in the Prevention of Disabilities due to Lymphatic Filariasis	April 13, 2010
AO 2010-0017	Guidelines in Surveillance and Response to Adverse Events Following Immunization (AEFI)	June 18, 2010
AO 2010-0018	Revised National Policy on Living Non-related Donation and Transplantation and its Implementing Structure Attending for the Purpose AO 2008-0004A	June 21, 2010
AO 2010-0024	Requirement of Permit to Construct in the Establishment of Free Standing Screening Drug Testing Laboratories Nationwide	July 29, 2010
AO 2010-0028	Policies and Guidelines in the Conduct of Human Immunodeficiency Virus (HIV) Counseling and Testing in Community and Health Facility	September 27, 2010
RA 10152	Mandatory Infants and Children Health Immunization Act of 2011	June 21, 2011